



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Pre-Deployment Screening - Psychosocial Document Date: 2022-03-30

Reset

Deployment Information

Operation: CFJSR	Region / Country: Kingston, ON
Departure Date: 1 July 2022	Length (months): 36

Family Demographics

Spouse: Yes	Number of Other Dependants: 2
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Explain how the member and the family feel about this deployment

Mbr requested this posting and as such welcomes this opportunity. He had a previous 4 year posting to a high readiness unit -- he and his family are accustomed to him being away and having short notice for them does not present significant issues. Mbr and his family are also happy to remain in the area and to not be posted out this year.

Detail any contact the member or the family had with Primary Care Clinicians or counselling service providers within the last 5 years pertaining to a personal problem or situation:

Who	Purpose	From (Date)	To (Date)	Resolved
Member	anger	Aug 2018	April 2020	Yes ☐ +

Comments Mbr indicated that he takes medication and utilizes various coping strategies. One of Mbr's son's engaged in supportive counseling during the past 5 years for issues related to school. This has resolved.

Describe if the member or the family is experiencing any psychological, emotional or social difficulties which may require the member to return early from this deployment

No issues identified.

Describe any difficulties with substance use, gambling or other addictions by the member or dependants

Mbr occasionally consumes alcohol and he uses cannabis daily. He indicated that it is not an issues to cease use and it does not impact his work. Mbr's partner occasionally consumes alcohol. She does not reportedly use any other substances. No use identified by Mbr's teen aged children.

Detail any legal matters that the member or dependants may be involved in

Nil as per Mbr.

Detail if there are any extended family members who are sick or suffering from a serious illness which prevents the member from being away at this time

Mbr's parents and in-laws are generally well and they live independently. There aren't any known serious illnesses that would prevent him from being away.

Detail any primary or substantial responsibility the member has to care for anyone other than the listed dependants

Nil as per Mbr.

Detail any other personal or family circumstances that would prevent the member from deploying at this time

No issues identified.



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Screening Status

I certify that I have conducted the psychosocial screening for this member and his / her dependants. Based on this screening, I assess that:

- ☒ GREEN: There are no known psychosocial considerations
☐ RED: There are known psychosocial considerations
☐ YELLOW: There are limitations related to psychosocial care

If Red or Yellow, check all that apply

- ☐ Substance Use or Behavioral Addictions
☐ Legal or Disciplinary
☐ Family Circumstances
☐ Personal
☐ Other:

Recommendations / Plan

Mbr has been dagged Green.

Signed by : Ms Tracey A Beckett, SOCW, KG-MH, 2022-03-30 08:46:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Pre-Deployment Screening - Psychosocial Document Date: 2022-03-30

END OF DOCUMENT

COMMENT

2022-03-30 13:17 Jennifer Noseworthy

30 March 2022: Verified mbr status, status is green. -JN



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Discharge Summary Document Date: 2020-07-14

CFHS GENERAL

Treatment Start Date : Tuesday, August 27, 2019

Date of Discharge : Friday, July 03, 2020

Reason for Discharge : Treatment completed

Status at Discharge : Improved

Diagnosis or Clinical Impression at Discharge : Mbr shares still having anxiety/anger moments but more capable of self-regulation with quicker return to baseline. He reports having full support of his wife and supporting her in her efforts through Covid-19 and working in NH.
Ok to close file with understanding he can reapply for counselling in the future as needed.

NARRATIVE

Summary of Treatment

Mbr and spouse requested couple therapy for issue in their relationship as both seemed to be living separate lives b/c she had issues with his expression of anger toward her and their kids. Both worked together on identifying the cause/source of anger both at home and at work according to Mbr. With help of Gottman Couple therapy techniques and Attachment theory they were able to better understand background of anger in core values. And in that understanding both partners were able to devise their own ways of being that worked for them to address anger issues as well as anxiety when things do not go his way; needing a break/pause and then return to discuss issues.

ASSESSMENT

Clinical Impression Classification : Family Circumstances : Relationship distress with spouse or partner

Operational Stress Injury : N/A

PLAN

RECOMMENDATION : Close file for couple therapy as per Mbr's request and completion of therapy

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2020-07-14 08:39:00

Signed off by : Lt Dakota Levesque, SOCW, PT-MH, 2020-08-26 09:38:00

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Admin Note Document Date: 2020-07-03

GENERAL INFORMATION

Mbr and spouse respond in good spirits stating all is well and continues to be well, they say.
Mbr confirms ok to close file with knowledge to reopen as needed.

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2020-07-03 08:05:00

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-06-02

Reset - Réinitialiser

Arrival Status - Ponctualité Other - Autre ☐	Session Took Place - Lieu de la session Phone - Téléphone ☐
Note :	Note :
Service Provided - Service offert Couple / Family - Couple / famille ☐	Session Number - Numéro de la session
Treatment Approach / Intervention - Approche de traitement / Intervention	
Main - Principale :	Solution Focused Therapy
Additional - Supplémentaire :	
Additional - Supplémentaire :	
Note :	
Narrative - Narration	
Both partners together for phone call Spouse talks first and reports actually doing very well with no issues at this time Mbr reports the same and adds doing well while he is not at work but anticipates increase of anxiety upon return to work Both partners state spending time together and actually enjoying it; Mbr shares being kinder to his wife and it actually working in his favor. Conversation opens to what Mbr and spouse want to do with the situation and couple therapy under present circumstances. Mbr shares wanting to touch base in July as he appreciates the contact. Although reminded of regular contacts with psychiatrist and psychologist, Mbr shares a sense of stability having regular check-ins.	
Current Status - Statut actuel Improved - Amélioration ☐	Outcome Measures - Mesures d'impacts
	OQ - 45 :
	Other - Autre :



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-06-02

Clinical Impression - Impression clinique

Mbr and spouse reports doing very well however with Mbr not working.
Good spirits; talkative and receptive to telephone conversation
Thought process clear with some anticipatory anxiety for when he returns to work; wife still works in NH

Clinical Impression Classification - Classification d'impression clinique

1: Relationship distress with spouse or partner

2:

3:

Note :

Recommendations / Plan - Recommandations / Plan

Couple doing well
Mbr agrees to monthly check-in July 2020

Patient Instructions - Instructions au patient

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2020-06-02 09:42:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-06-02

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09

Session Note Document Date: 2020-04-14

Reset - Réinitialiser

Arrival Status - Ponctualité Other - Autre ☐	Session Took Place - Lieu de la session Phone - Téléphone ☐
Note :	Note :
Service Provided - Service offert Couple / Family - Couple / famille ☐	Session Number - Numéro de la session
Treatment Approach / Intervention - Approche de traitement / Intervention	
Main - Principale :	Solution Focused Therapy
Additional - Supplémentaire :	Psychoeducation
Additional - Supplémentaire :	
Note :	
Narrative - Narration	
Wellness check with Mbr at home doing odd jobs around the house but also very bored, he says. He shares worry of his wife working in nursing home a lot and happy to see her return home every evening. He states looking forward to warmer weather for outdoor activities and being less cooped up. Mbr shares doing well when his wife is home but working a lot these days.	
Current Status - Statut actuel Slightly Improved - Amélioration légère ☐	Outcome Measures - Mesures d'impacts
	OQ - 45 :
	Other - Autre :



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-04-14

Clinical Impression - Impression clinique

Mbr reports being less reactive with anger; using self-reasoning with good effect
Good RRT voice
Good spirits with some level of worry over wife's working hours

Clinical Impression Classification - Classification d'impression clinique

1: Relationship distress with spouse or partner

2:

3:

Note :

Recommendations / Plan - Recommandations / Plan

Mbr is encouraged to call as needed
He shares plan to book an appointment once isolation lifts

Patient Instructions - Instructions au patient

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2020-04-14 13:39:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-04-14

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Admin Note Document Date: 2020-03-13

GENERAL INFORMATION

Called Mbr to cancel and reschedule appointment for following week with good effect.

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2020-03-13 07:57:00

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-02-25

Reset - Réinitialiser

Arrival Status - Ponctualité Was on time - Ponctuel ☒	Session Took Place - Lieu de la session Office - Bureau ☒
Note :	Note :
Service Provided - Service offert Couple / Family - Couple / famille ☒	Session Number - Numéro de la session
Treatment Approach / Intervention - Approche de traitement / Intervention	
Main - Principale : Emotionally Focused Therapy	
Additional - Supplémentaire :	
Additional - Supplémentaire :	
Note :	
Narrative - Narration	
Mbr and spouse attend session He shares not remembering the conversation last session but when given time he recaps in fair order. Spouse shares of him often turning to ignorance when overwhelmed and is usually easily overwhelmed. He admits still hiding/withholding from her in their conversations but does not do it on purpose. Conversation opens to observations from last session in his behavior over the week. Mbr shares wanting more patience and needing it when dealing with middle son not doing his homework. Observations shared that even just wanting more patience shows compassion and the need for compassion when dealing with his son. He states not seeing things from a positive point of view: he is asked if it is by habit or for a fact he cannot see the positive in him; behavior/feelings/thoughts. Mbr shares by habit and not conditioning himself to think positive throughout his adulthood but rather choosing to think through the negative never really getting to any positive, he replies. Instructions follow on the habit of negativity and the possible fear of responsibility to change. And furthermore, the idea may be daunting at this point in therapy; Mbr does not agree b/c saying he usually does not shy away from responsibility, realizing responsibility toward others. Review of responsibility toward himself would mean to understand and know his positive points as well as his challenges and mistakes equally, he acknowledges.	
Current Status - Statut actuel Slightly Improved - Amélioration légère ☒	Outcome Measures - Mesures d'impacts
	OQ - 45 :
	Other - Autre :



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-02-25

Clinical Impression - Impression clinique

Good session with marked progress in openness; this time without the added visible disgruntled effort in hearing through a session telling him of his choice to not know his positives now that they are pointed out.
He readily identifies given time values of compassion from last session and humor; remarkable given his persistence in not knowing himself.
Mbr reports confusion, frustration and anger at times with good effect in learning something he never paid attention to and is now required for the benefit of couple therapy
thought process of overgeneralization is common and addressed allowing him to maintain focus on him
Spouse shares observations of shifting toward more positive conversation; he still does not identify this as a plus but states it is only the obvious and shrugs it off

Clinical Impression Classification - Classification d'impression clinique

1: Relationship distress with spouse or partner

2:

3:

Note :

Recommendations / Plan - Recommandations / Plan

Continue with spouse witnessing his interactions with clinician and revealing to him his usual demeanor when they react/ interact together; she is visual learner and he is doing.
The more he concentrates on describing issues and/or positive from his frame of reference the more he can shift to enactment with his wife with greater ease and less distress

Patient Instructions - Instructions au patient

Mbr is invited to think of something he may have done/thought/said that is so bad that he cannot see himself ever in a positive light; thus refusing the positive out of undeserving sentiments. Mbr shares possibly not ready for this advancement in self-reflection; instructions follow on the fact that he identifies level of readiness is proof enough he is ready as he is able to self-evaluate; he may also be able to identify and describe himself.

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2020-02-26 07:59:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-02-25

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-02-18

Reset - Réinitialiser

Arrival Status - Ponctualité Was on time - Ponctuel ☒	Session Took Place - Lieu de la session Office - Bureau ☒
Note :	Note :
Service Provided - Service offert Couple / Family - Couple / famille ☒	Session Number - Numéro de la session
Treatment Approach / Intervention - Approche de traitement / Intervention	
Main - Principale :	Emotionally Focused Therapy
Additional - Supplémentaire :	Psychoeducation
Additional - Supplémentaire :	
Note :	
Narrative - Narration	
Mbr attends session with spouse He brings homework and addresses the positive in his achievements. He shares wondering why focusing on the positive is so important when he knows he is sub-standard in self-evaluation His wife shares having an easier time seeing his positives as well as identifying and describing her values and positive points. Conversation opens to doing values clarification in session; his wife is provided with a list and he has to identify his values. Wife states seeing the exercise as duty for him and him not taking it seriously b/c of not understanding the importance. Instructions follow on positive points clinician identifies in him since beginning therapy; 5 values are identified with accuracy. He is then invited to recall a situation that he seemingly handled with sense of 'well-done'; after his narrative he is encouraged to identify which values are proof he possesses and are evident in his thoughts and behavior in this situation. He identifies 2 only. His wife however responds in identifying at least 4 as humor was not in the situation with good judgment. Discussion further reveals the importance in him to identify and describe values and beliefs in order to get a more accurate assessment of positives in his life and help shift away from overwhelming negativity. Wife shares her observations and concurs with values identified in him. Mbr shares work necessary to shift negative perspective and habit of jumping to the negative first.	
Current Status - Statut actuel Slightly Improved - Amélioration légère ☒	Outcome Measures - Mesures d'impacts
	OQ - 45 :
	Other - Autre :



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-02-18

Clinical Impression - Impression clinique

Mbr shows marked improvement in grasping concepts like values clarification for a clearer identity, worth and enjoyment
The presence of his wife in a witnessing role allows him to better understand importance he often misses r/t improving self-knowledge
Mood/affect: slight anger when learning new concept like identity; easy to calm and review learning curve; stable and open with candid self-disclosure
Thought process: still jumping to negative conclusions, catastrophizing, minimizing his qualities/talents, low self-worth
Mbr shares benefiting from seeing where values come visible in his work/home with help from his wife

Clinical Impression Classification - Classification d'impression clinique

1: Relationship distress with spouse or partner

2:

3:

Note :

Recommendations / Plan - Recommandations / Plan

Continue with present module;
- review values and describe beliefs that r/t each value
- review cognitive errors and ways to shift away from them in restructuring thoughts

Patient Instructions - Instructions au patient

Mbr is encouraged to maintain positivity; thoughts, mood and reactions with examples provided
- to write down beliefs system r/t values addressed in session
- share thoughts/ideas with his wife; wife very appreciative of him talking more with her

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2020-02-19 09:10:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-02-18

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Periodic Case Review Document Date: 2020-02-05

CFHIS GENERAL

Treatment Start Date : Tuesday, August 27, 2019

Diagnosis or Clinical Impression

Mbr and spouse continue to request couple therapy in view of progress Mbr is making with review of background/childhood experiences and their impact on his relationship with his wife. Taking the time to slow thought process and emotionality to pinpoint turning point in his life when he acknowledges anger/guilt/shame/fear.

NARRATIVE

Summary of Treatment : Mbr and spouse continue to attend session in couple therapy. In a combination of CBT/EFT/ACT therapy modules both partner show increase in understanding their way of being within their relations and gradually identify areas of challenge and a request for assistance.
Process is slow b/c of possible resistance on part of Mbr however addressing strong emotions seems to help unlock sensitivity with increasing self-knowledge as expressed by Mbr.

ASSESSMENT

Clinical Impression Classification : Family Circumstances : Relationship distress with spouse or partner

Current Status : Unchanged Operational Stress Injury : N/A

Treatment Approach / Intervention

Main : Emotionally Focused Therapy

Additional : Cognitive Behavioral Therapy

Additional : Other

Note : Elements of ACT

PLAN

RECOMMENDATION : Continue with information sharing and exploration of thoughts/reactions and strong emotions experienced by both partners throughout their marriage.
- focus on strong emotions and their link to reactions; his structured idealistic view to her easy going demeanor

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2020-02-05 08:43:00

Signed off by : Capt Krista J Erickson, SOCW, PT-MH, 2020-02-05 10:39:00

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09

Session Note Document Date: 2020-02-04

Reset - Réinitialiser

Arrival Status - Ponctualité Was on time - Ponctuel ☒	Session Took Place - Lieu de la session Office - Bureau ☒
Note :	Note :
Service Provided - Service offert Couple / Family - Couple / famille ☒	Session Number - Numéro de la session
Treatment Approach / Intervention - Approche de traitement / Intervention	
Main - Principale :	Cognitive Behavioral Therapy
Additional - Supplémentaire :	
Additional - Supplémentaire :	
Note :	
Narrative - Narration	
Mbr and spouse attend session He states still being in a place of confusion; stuck point unknown, emotions all over the map and thoughts of what therapy may uncover. Mbr reports doing homework and using present events to describe in ABC model; he states analyzing 2 scenarios. Spouse shares seeing in the anxiety in him but letting his figure it out on his own. Conversation opens to recap of last session from childhood age 8 or grade 4: thoughts himself as "stupid", hospitalized 2 months for SA, onto age 14-16 when he decided to hate the world. Mbr agrees in keeping the "hate" alive with valid events only to show formulation of cognitive structure in present ideology as highly negative with very little positive world view or of himself. Mbr admits to struggling to find positive moments throughout the age range addressed to date 8-16. Spouse states as witnessing her husband and the conversation with the clinician, she sees her husband as argumentative and stuck on his way of thinking but also his way of answering questions about himself he was never pushed until now to reflect and answer. She acknowledges the discomfort, increase in anxiety and shame in admitting his strong emotions in his way of thinking.	
Current Status - Statut actuel Unchanged - Inchangé ☒	Outcome Measures - Mesures d'impacts
	OQ - 45 :
	Other - Autre :



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-02-04

Clinical Impression - Impression clinique

Mr admits never taking his childhood apart and viewing it from a psychological point of view
He acknowledges stuck point around age 16 with presence of fierce moral judgment/own view of cultural norms/influence of background with parents, educations and peers; duality in highly negative view yet strong sense of self and ideology
Mood/affect: angry, sad, ashamed and humourous at times however congruent with topic of conversation
Thought process less confused than anticipated; good self-reflection; limited insight at this point
Mr denies SI/VI
He shows more affection and loving glances at his wife not observed previously

Clinical Impression Classification - Classification d'impression clinique

1: Relationship distress with spouse or partner

2:

3:

Note :

Recommendations / Plan - Recommandations / Plan

Continue with combination CBT/ACT/EFT for strong emotions/reactions
- review accomodation and assimilation in congitive development in adulthood
- explore positive points he was to identify and describe
- intro of congitive reconstruction; purpose, function and desired outcome

Patient Instructions - Instructions au patient

Mr is invited to start writing his thoughts and emotions in a journal of his experience in therapy and other events that bring out strong emotions; Mr agrees and wife states to provide journal

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2020-02-05 08:32:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-02-04

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-01-30

Reset - Réinitialiser

Arrival Status - Ponctualité Was on time - Ponctuel ☒	Session Took Place - Lieu de la session Office - Bureau ☒
Note :	Note :
Service Provided - Service offert Couple / Family - Couple / famille ☒	Session Number - Numéro de la session
Treatment Approach / Intervention - Approche de traitement / Intervention	
Main - Principale :	Cognitive Behavioral Therapy
Additional - Supplémentaire :	
Additional - Supplémentaire :	
Note :	
Narrative - Narration	
Mbr and spouse attend session They share looking forward to today's session as it is to go deeper into the emotionality Mbr holds onto. He states fear of going back to work when emotionally stirred from session; he suggests they book in the afternoon from now on, wife agrees. Conversation opens to importance of self-care practices for both partners on their own. Mbr describes using the gym and reading while she states having friends and family to talk with when on overload. Mbr is invited to talk of his first memories of feeling unloved, odd, weird as per his narrative; Mbr obliges His wife witnesses his narrative and shows surprise at the deeper emotions he shares; Mbr states being the first time he shares openly without restraint with her in therapy. She states knowing the deeper anger he holds and often shows her but not knowing the source; bullied and then being the bully as a child between 7-15 years of age approx, he attests. Both share their thoughts and feelings on his first session that allows Mbr and his wife to disclose deeper thoughts and feelings as they arise and as per their memories; she attests being the better memory to corroborate his stories as much as possible.	
Current Status - Statut actuel Unchanged - Inchangé ☒	Outcome Measures - Mesures d'impacts
	OQ - 45 :
	Other - Autre :



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-01-30

Clinical Impression - Impression clinique

Mbr shows openness, motivation and readiness to address resistance he felt with expressing, talking and identifying emotions
Mood/affect: angry, anxious with much movement in his chair when identifying just how angry he felt as a child and reliving it while he recounts the memories.
Thought process clear and easily redirected when moving away from a topic
Mbr denies SI/VI however angry he feels
He shares being calmer by end of session but still unable to return to work with such significant weight of emotions

Clinical Impression Classification - Classification d'impression clinique

1: Anger

2: Relationship distress with spouse or partner

3:

Note :

Recommendations / Plan - Recommandations / Plan

Mbr and wife book for afternoon appointment to have the opportunity to go home after session
Continue with exploration of intense emotion of anger and expressing it through bullying
- review ways his behavior impacts him as an adult; personal life with limited friends, married life mired with difficulties throughout as per his wife and his work life where he shares impression of people being afraid of him

Patient Instructions - Instructions au patient

Self-care practices to soothe and self-soothe; both share having a good talk after session to review the impact on each other

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2020-01-31 08:22:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-01-30

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-01-24

Reset - Réinitialiser

Arrival Status - Ponctualité Was on time - Ponctuel ☒	Session Took Place - Lieu de la session Office - Bureau ☒
Note :	Note :
Service Provided - Service offert Couple / Family - Couple / famille ☒	Session Number - Numéro de la session
Treatment Approach / Intervention - Approche de traitement / Intervention	
Main - Principale : Emotionally Focused Therapy	
Additional - Supplémentaire :	
Additional - Supplémentaire :	
Note :	
Narrative - Narration	
Mbr and spouse attend session saying they had a good holiday and are doing fine. However Mbr states having moments of depression with fleeting thoughts of suicide and difficulty shaking it off. She shares not knowing this and would not have been able to help as she was busy with family, full household and children. Mbr further goes into enactment with his wife listening to what he has to say; he talks directly about deep-set emotions and thoughts without interruption as his wife silently listens. After talking approx 2 minutes without pause and after stating he had nothing to say to her, he opens purported source of his pain/hurt. Mbr admits truly believing he is not loved by anyone and questions anyone who says they love him even his wife and not knowing why people do not love him, he believes to be unlovable. Conversation opens to intense emotionality of anger, sorrow, despair; Mbr swiftly changes the topic and noticed by wife who mentions his discomfort talking intense emotions. Mbr shares never opening up to this extent to anyone and being allowed to spew off without interruption seems to have uncovered a darker place, he says. He shares not wanting to go there b/c of the fear of bringing back depressive symptoms and thoughts of suicide. But, he also verbalizes beginning to understand why he has not made progress with the amount of therapy he is getting. He describes a rawness, vulnerability and almost weakness. Instructions follow on having his permission to proceed in unveiling further self-discoveries with same intervention; Mbr agrees if it will give him peace within himself. Wife shares the impact of hearing him at that point of intense hurt/sorrow. She shares helplessness in knowing how to help and what to say but if just being there helps him feel supported then she will stay the session to hear him out; both partners agree to return to topic of primary emotion of hurt/pain next session.	
Current Status - Statut actuel Unchanged - Inchangé ☒	Outcome Measures - Mesures d'impacts
	OQ - 45 :
	Other - Autre :



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-01-24

Clinical Impression - Impression clinique

The intensity of the session opened the last resistance to therapy and allowed the Mbr to disclose what he could not explain under normal circumstances.

Mood/affect: serene, candid after the discussion but unease prior with reports of depressive symptoms

Thought process: clear once allowed to speak freely; he revealed many realizations after his brief rant

Mbr denies SI/VI

He shares finding in his wife the best support and therefore wants her present when he revisits primary emotion

Clinical Impression Classification - Classification d'impression clinique

1: Relationship distress with spouse or partner

2:

3:

Note :

Recommendations / Plan - Recommandations / Plan

Continue with EFT to further describe emotions; Mbr to identify and describe while his wife will listen and then share impact of his disclosure

Patient Instructions - Instructions au patient

Both partners are to seek soothing and self-soothing practices that help them stay grounded; for him to get out of place of comfort when going through depressive symptoms and for her ways to maintain emotional stability and secure within herself, she mentions her work and the children

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2020-01-24 12:41:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2020-01-24

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-12-18

Reset - Réinitialiser

Arrival Status - Ponctualité Was on time - Ponctuel ☒	Session Took Place - Lieu de la session Office - Bureau ☒
Note :	Note :
Service Provided - Service offert Couple / Family - Couple / famille ☒	Session Number - Numéro de la session
Treatment Approach / Intervention - Approche de traitement / Intervention	
Main - Principale :	Cognitive Behavioral Therapy
Additional - Supplémentaire :	
Additional - Supplémentaire :	
Note :	
Narrative - Narration	
Mbr and spouse attend session Both reports doing well; spouse shares seeing efforts and changes in her husband specifically in regulating his temper. He shares still not seeing marked improvements; not knowing how to be a regular human being, he says Conversation opens to comments Mbr addressed in his narrative being negative and self-derogatory; he agrees to follow instructions. With help from exercise 'restructuring thoughts' from CBT, he analyzes one of the recent situations addressed as highly negative. Spouse will witness his response and pay particular attention to the positive she sees in him actually doing/saying, spouse agrees. Good session in keeping Mbr on topic, in the present and genuine with self-disclosure; he shares not seeing himself as a liar and not liking that he is seemingly not telling the truth. Ownership/responsibility/accountability are addressed to allow him to see a truth; he knows more than he lets on and becomes flustered when pushed to think for himself, he surmises. He admits going to the easier answer in feigning to not know rather than think through something that he is not used to thinking liking thinking more positive about himself. Spouse shares observations of his efforts and knowing more than he lets on as well as the awareness he has of self-improvement but b/c he does not like to brag he would rather go to the negative instead. Brief instructions follow on receiving and giving compliments as well as focusing on self to work more in-depth through proper observation; if it is positive then it is positive, own it, he finds this difficult at present, he says.	
Current Status - Statut actuel Slightly Improved - Amélioration légère ☒	Outcome Measures - Mesures d'impacts
	OQ - 45 :
	Other - Autre :



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-12-18

Clinical Impression - Impression clinique

Mbr shows increasing receptivity in understanding his way of being linking to problematic thoughts of self
The exercise opened learning accuracy of expression, authenticity and ownership/responsibility/accountability
Spouse realizes also shifting her way of thinking when he cannot grasp a concept but knowing he can and believing in him
Candid, open and receptive to addressing issues directly
Thought process self-diminutive seems more comfortable than telling himself he can and is better than what he lets on
Both report having a good informative session

Clinical Impression Classification - Classification d'impression clinique

1: Relationship distress with spouse or partner

2:

3:

Note :

Recommendations / Plan - Recommandations / Plan

Mbr takes home handout to practice restructuring thoughts exercise and is encouraged to write down his thoughts, feelings and behaviors in accuracy of expression

Continue with couple therapy; they share talking more and need help better understanding one another as they seem to be doing in the past 2 sessions, they say

They book their next appointment for January 2020

Patient Instructions - Instructions au patient

Self-care practices; writing down his thoughts/feelings/behavior and filling out practice sheet to help shift self-judgment to a more positive tone

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2019-12-18 10:00:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09

Session Note Document Date: 2019-12-18

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-12-02

Reset - Réinitialiser

Arrival Status - Ponctualité Was on time - Ponctuel ☒	Session Took Place - Lieu de la session Office - Bureau ☒
Note :	Note :
Service Provided - Service offert Couple / Family - Couple / famille ☒	Session Number - Numéro de la session
Treatment Approach / Intervention - Approche de traitement / Intervention	
Main - Principale : Emotionally Focused Therapy	
Additional - Supplémentaire :	
Additional - Supplémentaire :	
Note :	
Narrative - Narration	
Mbr and spouse attend session Mbr get right into negative view of himself with depreciative comments from his narrative; i am a bad person... He shares not believing all the time but most of the time with my wife. He shares wanting to tell his wife something in a safe place so as to prevent him from hurting her with his words as per his usual. He proceeds to tell her his thoughts, emotions and reactions to actions previously with their son. Keeping the conversation on him, he is invited to share his thoughts and values of the issue, what is important to him and the background info unfolding to this moment. He graciously shares and discloses thoughts and feelings as each word and meaning used in his conversation with his wife is slowed down for his pursuit. Emotions and thoughts are more fully viewed from what he previously thought as 'bad' parenting on the part of his wife. He shares realizing the transfer of emotions and thoughts to his son when the probability of his son actually thinking and feeling the emotions he describes may be slim b/c he really does not know for sure, he assures. Delving into the raw emotions makes him so uncomfortable that he moves in his chair and looks upward so as not to show/express emotions. With this observation he is made aware of his reactions to strong emotions when he allows himself to feel even begrudgingly as in today's session. He reports feeling physically and emotionally drained and the rest of the session is spent in light conversation about the book he reads on Attachment theory with good effect. His wife shares after witnessing the session that she has never seen him react emotionally with anything but anger and was pleasantly surprised getting to see another side of possibilities. She shares seeing the difficulty he has in expressing, identifying and describing emotions when in effect it comes easy for her; she realizes just how difficult this is for him and his willingness to put himself through it for the benefit of their relationship. She shares knowing she does not want to give up on them.	
Current Status - Statut actuel Unchanged - Inchangé ☒	Outcome Measures - Mesures d'impacts
	OQ - 45 :
	Other - Autre :



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-12-02

Clinical Impression - Impression clinique

In this 6th session, Mbr finally acknowledges the importance of addressing his emotionality stemming from childhood and affecting his present state of adulthood. It is as if he sees emotions and reactions/thoughts for the first time as they are within himself without cover-up or adjusting.

Mood/affect: terrified, angry at being purportedly made to see the benefit of therapy and allowing his wife to witness his reactions/emotions and thoughts; he calms near end of session with light banter

Thought process; seen in today's session as per his narrative/behavior: projecting his thoughts/feelings on his son, dramatizing out of proportion as he recalls incidents from childhood; personalizing other's thoughts and feelings as if he is to blame or the instigator of them

Mbr leaves in good spirits knowingly with the loving support of his wife

Clinical Impression Classification - Classification d'impression clinique

1: Relationship distress with spouse or partner

2:

3:

Note :

Recommendations / Plan - Recommandations / Plan

Continue with EFT: focus on his emotionality left unsaid and as it is expressed allowing his spouse to witness once more He shares understanding her ease with strong emotions and articulation; he admits needing help in that area and willing to have her witness him in session

Try enactment where he expresses to his wife with accuracy of expression a short dialogue
Identifying and describing bodily changes when anxious, angry to better corner them

They book next appointment in 2 weeks

Patient Instructions - Instructions au patient

Observe his way of being when angry and anxious when trying to cover-up his emotionality

Feel free to simply be with his son to talk as with his son with no pressure to script or check-in and just talk with him; he shares willing to give it a try

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2019-12-03 11:13:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-12-02

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Periodic Case Review Document Date: 2019-11-22

CFHS GENERAL

Treatment Start Date : Tuesday, August 27, 2019

Diagnosis or Clinical Impression : Mbr and spouse only begin work on identifying and describing negative pattern, primary/secondary emotion, extent of attachment injuries and the impact on their relationship. Both partners seem to become more comfortable in the revelation of 'dark' emotions Mbr experiences.

NARRATIVE

Summary of Treatment : Mbr and spouse request couple therapy to help with reconnecting. However Mbr did consider separation and spouse would have agreed given the status at the time of initial assessment. At present and using a combination of Gottman/EFT, they begin to identify and describe the extent of impact of longstanding deep negative emotions on their relationship. Both continue to fully participate bi-weekly and complete assignments as required.

ASSESSMENT

Clinical Impression Classification

Family Circumstances : Relationship distress with spouse or partner

Personal : Anger

Current Status : Unchanged Operational Stress Injury : N/A

Treatment Approach / Intervention

Main : Emotionally Focused Therapy

Additional : Other

Additional : Supportive Counselling

Note : Gottman Therapy for couples

PLAN

RECOMMENDATION : Continue with focus on first stage in EFT and id/describe 4 horsemen in Gottman.

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2019-11-22 07:43:00

Signed off by : Capt Krista J Erickson, SOCW, PT-MH, 2019-11-24 17:10:00

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-11-21

Reset - Réinitialiser

Arrival Status - Ponctualité Was on time - Ponctuel ☒	Session Took Place - Lieu de la session Office - Bureau ☒
Note :	Note :
Service Provided - Service offert Couple / Family - Couple / famille ☒	Session Number - Numéro de la session
Treatment Approach / Intervention - Approche de traitement / Intervention	
Main - Principale :	Emotionally Focused Therapy
Additional - Supplémentaire :	
Additional - Supplémentaire :	
Note :	
Narrative - Narration	
Mbr and spouse attend session Both were talking before entering the office. Mbr states joking-like not appreciating her purportedly curt remarks. She states they were just joking around nothing serious about a new puppy they have. He shares being offended by her comments and talks about the emotions triggered by the incident and recall of past hurts by her. He unveils in his narrative the extent of feeling hurt by her comments and opens the discussion to the anger/rage/hatred that seem to emerge at this moment. Being flooded, he takes a few minutes to breathe while his spouse is instructed on being witness to his emotions/thoughts/reactions as well as the impact they have on her. Both oblige as he returns to his narrative keeping topic on him, his feelings, thoughts and reactions however difficult. Conversation opens to sharing their perception of a seemingly light conversation prior to session. She speaks to him giving him permission to talk openly; she observes the rawness with which he continues and reports satisfaction at finally talking of the deeper emotions she knew he harbored. Both state feeling comfortable enough to share the depth of the topic. In this 2-hour session, both partners express satisfaction in talking openly about longstanding negative emotions that impact their relationship. Both take their turn at disclosing the impact of these negative emotions and the desired outcome. Instructions follow on depth of exposure in self-disclosure, vulnerability toward anything perceptively negative and the need for validation. Both share after waiting long enough to address issues of disconnect b/c of highly negative emotions in the way of their efforts to reconnect. They mutually express the desire to move forward in therapy. Further instructions follow on effective use of EFT in address negative cycle focusing on the strong emotions reveals in today's session; both partners agree to continue.	
Current Status - Statut actuel Unchanged - Inchangé ☒	Outcome Measures - Mesures d'impacts
	OQ - 45 :
	Other - Autre :



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-11-21

Clinical Impression - Impression clinique

A breakthrough happened in revelation of the deep dark emotions he calls black emotions of anger/rage/hatred felt seethingly most days
Both partners agree to continue couple therapy addressing them; she expresses relief at finally getting somewhere, he shows willingness, readiness and motivation to continue
Mood/affect: mixed with anger, relief, sadness, embarrassment and regrets
Thought process; clear and goal-directed for both partners
Once he opened the conversation and felt comfortable enough to continue, he seemingly did not hold back in articulation that revealed genuineness and depth of suffering for both partners
Mbr and spouse deny risk of harm to self or others

Clinical Impression Classification - Classification d'impression clinique

1: Relationship distress with spouse or partner

2: Anger

3:

Note :

Recommendations / Plan - Recommandations / Plan

Continue conversation using EFT; review negative pattern, attachment injuries, sense of loss
Introduce way to effectively validate one another

Patient Instructions - Instructions au patient

Both partners are highly encouraged to self-soothing and soothing one another with acts of kindness for the next 2 weeks in light of work ahead in addressing a foreboding enemy
Mbr is invited to begin a gratitude journal to help with maintaining a positive perspective while exploring the negative; Mbr agrees to write down 20 a day for 6 days and then only write down his choice of things/person for which he is highly grateful

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2019-11-22 07:31:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09

Session Note Document Date: 2019-11-21

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-10-29

Reset - Réinitialiser

Arrival Status - Ponctualité Was on time - Ponctuel ☒ Note :	Session Took Place - Lieu de la session Office - Bureau ☒ Note :
Service Provided - Service offert Couple / Family - Couple / famille ☒	Session Number - Numéro de la session
Treatment Approach / Intervention - Approche de traitement / Intervention	
Main - Principale : Emotionally Focused Therapy	
Additional - Supplémentaire : Other	
Additional - Supplémentaire :	
Note : Gottman: 4 Horsemen	
Narrative - Narration	
Mbr and spouse attend session They state doing better in communicating more of pertinent information as well as better listening skills, Mbr shares. She shares however gradually opening more to him and at times not knowing what to say to him. He shares wanting her to work on communication with the children as it does not meet his standard of where he is at in his healing process. Conversation opens to attachment style and injuries; he shares possibly having an injury because of upbringing. Discussion allows both partners to hear where the other is at in adjusting/adapting to stress in general but also ways that already exist that help them along. She shares seeing the secure attachment but also the hesitation to open to him because of past hurt/pain. He states being anxious on a regular basis and always watching out not to fall into old habits, he shares being exhausted mentally and unable to take on anything else to address. Instructions follow on shifting the negative pattern to focus on what he does want to see in his behavior. He shares the difficulty in seeing the positive because of longstanding habit to first see the negative, deal with it and never really getting to any positive.	
Current Status - Statut actuel Unchanged - Inchangé ☒	Outcome Measures - Mesures d'impacts
	OQ - 45 :
	Other - Autre :



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-10-29

Clinical Impression - Impression clinique

Both continue to show openness, willingness and motivation to attend couple therapy
Mood/affect: good humor, pleasant and serious in addressing issues
Thought process; clear and goal-directed
Thought content coherent with focus on self-improvement
Both state not seeing issues from an attachment perspective; for him seeing the negative before any positive and her feeling secure within herself but struggling when it comes to addressing his issues b/c dealign with negativity most of the time

Clinical Impression Classification - Classification d'impression clinique

1: Relationship distress with spouse or partner

2:

3:

Note :

Recommendations / Plan - Recommandations / Plan

Continue with couple therapy integrating attachment theory with behavior in 4 horsemen
- intro Rapoport Intervention keeping in mind to identify 4 horsemen on their own and rephrase as needed

Patient Instructions - Instructions au patient

Self-care practices: maintaining playfulness and kindness and keeping conversations light for now

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2019-10-30 07:46:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09

Session Note Document Date: 2019-10-29

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-10-11

Reset - Réinitialiser

Arrival Status - Ponctualité Was on time - Ponctuel ☒	Session Took Place - Lieu de la session Office - Bureau ☒
Note :	Note :
Service Provided - Service offert Couple / Family - Couple / famille ☒	Session Number - Numéro de la session
Treatment Approach / Intervention - Approche de traitement / Intervention	
Main - Principale :	Other
Additional - Supplémentaire :	
Additional - Supplémentaire :	
Note : Attachment theory	
Narrative - Narration	
Mbr presents alone as invited to discuss his way of being in the coupleship He shares doing better in self-regulation of anger: no freaking out or wallowing, he says could be medication that helps sort his way through. Conversation opens to values that are seemingly revealing in his narrative; he shares seeing compassion and understanding what his wife went through. Instructions follow on attachment theory and the anxiety/avoidance that seem to stabilize with a more secure sense of self. He admits feeling more secure but but also riddled with guilt at what his behavior caused in the past, he says. He shares realizing his choices and resiliency now feeling better about himself. He shares thinking guilt as his primary emotion, however on closer review he understands fear as a driving force. He shares thinking more clearly and being open to learning from his reactions; he states taking away today his intelligence to think through his sense of being to sift through his desired outcome.	
Current Status - Statut actuel Slightly Improved - Amélioration légère ☒	Outcome Measures - Mesures d'impacts
	OQ - 45 :
	Other - Autre :



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-10-11

Clinical Impression - Impression clinique

Mbr shows good understanding of DBT principles of core mindfulness; ways to identify and describe wise mind when reacting; he shows good insight and ability to think through his emotions/reactions/thoughts
Mood/affect: upbeat, receptive to learning from attachment theory and ways he is returning to secure sense of self
Thought process clear and goal directed
Thought content on self-improvement and making his life better with his wife

Clinical Impression Classification - Classification d'impression clinique

1: Relationship distress with spouse or partner

2:

3:

Note :

Recommendations / Plan - Recommandations / Plan

Mbr appreciated coming in and receiving 1:1 on attachment theory
Continue with couple therapy with focus on negative cycle and ways to shift the negativity
Open conversation on a topic of their choice using stress reducing conversation

Patient Instructions - Instructions au patient

Mbr is given praise to continue to work on doing what he is doing that helps maintain a sense of security, resilience and choices

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2019-10-15 07:13:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-10-11

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-10-01

Reset - Réinitialiser

Arrival Status - Ponctualité Was on time - Ponctuel ☒	Session Took Place - Lieu de la session Office - Bureau ☒
Note :	Note :
Service Provided - Service offert Couple / Family - Couple / famille ☒	Session Number - Numéro de la session 2
Treatment Approach / Intervention - Approche de traitement / Intervention	
Main - Principale : Other	
Additional - Supplémentaire : Emotionally Focused Therapy	
Additional - Supplémentaire :	
Note : Gottman Feedback; EFT Negative Pattern	
Narrative - Narration	
Mbr and spouse attend session They share no drastic changes in relationship but noticing some differences: he shares seeing her efforts, she shares seeing him to be a lot nicer toward her. Both state however finding argument in parenting style; he believes she guilts their son in doing things, while she does not see the same way but only agrees with him to close the argument. Both are encouraged to have the argument in session today while their responses are recorded on whiteboard to outline the negative pattern. Both partners show heightened responses however she remains outwardly calmer; he admits accusatory opening in argument and she admits shutting down soon into listening to him. Both identify fears that drive underlying reactions: his fear in people seeing him as a failure and she fears not being enough ever. They openly talk of the impact on the conversation in maintaining 4 horsemen; criticism/defensive cycle, they attest. Both acknowledge having a good session exposing moment in time when both are seemingly at their worse.	
Current Status - Statut actuel Unchanged - Inchangé ☒	Outcome Measures - Mesures d'impacts
	OQ - 45 :
	Other - Autre :



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-10-01

Clinical Impression - Impression clinique

Both partners continue to show receptivity to outlining negative pattern; addressing their way of being and possible stuck point

Mood/affect: Mbr shows high anxiety but good tolerance in hearing/seeing comments; she shows receptivity to exposure of their way of being when arguing; different when seeing it on the whiteboard; both remain open, willing and ready to address stuck points

Thought process clear and logic; thought content on seeing the stuck points that exist in their arguments

Mbr shows emotional exhaustion and admits to trying so hard to change certain areas he recognizes feeling overwhelmed

Clinical Impression Classification - Classification d'impression clinique

1: Relationship distress with spouse or partner

2:

3:

Note :

Recommendations / Plan - Recommandations / Plan

Continue with couple therapy:

- review 4 horsemen and ways to address shifting them to more effective communication
- review attachment theory and ways of boundary failure in bonding with another person r/t childhood experiences
- explore meta-emotions: recognize the discomfort and reduce flooding

Patient Instructions - Instructions au patient

Self-care practices: for him to put aside intense desires to change himself and just be himself with her in playfulness/romance; for her to share more of herself in thoughts, feelings and desires however seemingly phony at first, she admits.

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2019-10-02 10:28:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09

Session Note Document Date: 2019-10-01

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Post-Deployment Assessment Document Date: 2019-09-19

Reset

Deployment Information

Tracking Number: 225051 Number of Months Since Deployment: 12
Primary Care Clinician or CDU: CDU-2

Deployment Experience

Mbr shared that this deployment sent him into a downward spiral in regards to mood, personal issues, difficulty with work. Mbr shared that he requested to leave the unit after coming back from this deployment and because of what transpired during and following this deployment he has submitted his VR from the CAF. He shared that one of the primary issues on his most recent deployment (approx. one year ago) was interpersonal issues with his CoC. Mbr shared that they were overworked, that he was ridiculed at times, and reported overall poor treatment by his immediate CoC.

Mental Health Status

Sleep

Mbr reports that sleep is okay at present, can fall asleep and is able to stay asleep. Does not wake feeling rested however.

Appetite

Mbr reported losing 65 lbs in one month following this deployment, commenting that he thinks rel'd to his ceasing of alcohol consumption. He described his appetite as normal currently, with two meals per day, does not eat breakfast as he does intermittent fasting.

Mood

Mbr described his mood as "super shitty", very anxious at times, also periods of low mood, states following deployment as he is mood deteriorated engaged in self-doubt, self-deprecation. He has linked with psychiatry since his return and has had his medication adjusted a couple of times. Mbr shared that he had been seeing Psychologist at CSOR prior to deployment and doesn't think he was in the "right frame of mind" to see some of the things he saw during this deployment. Mbr did not identify any acute/imminent risk issues during this screening interview, but did endorse "thoughts that he would be better off dead, or hurting himself in some way" in the Enhanced Post Deployment Screening Booklet.

Cognitive Functioning

Mbr described poor concentration, decreased focus, stated that he was unable to focus on work as he was too preoccupied with what others were thinking of him.

Substance Use

Mbr shared that after this deployment he stopped drinking and is now using marijuana (approx. one gram per day) to help with anxiety. Mbr shared that prior to his ceasing drinking he would have typically consumed approx. 24 tallboys in a week. States now he has maybe consumed a case of beer over the past year.

Personal Relationships

Spouse

Mbr shared that he and his spouse have engaged in couple therapy. Mbr shared that following his return from deployment, with the decline in his mood, this started to negatively affect the rel'p with his spouse and they have since sought out couple therapy.

Family

No significant changes/issues with other family members.

Interpersonal Relationships



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Post-Deployment Assessment Document Date: 2019-09-19

Work

Mbr reported ++interpersonal conflict with CoC while deployed, which had a negative impact on mbr's mood, resulted in a significant shift in his ability to trust CoC, to the point where mbr requested to leave the regiment following return from deployment.

Social

Mbr described poor social support network. States he had friends before joining CSOR regiment but had falling out and is now more socially isolated. Mbr states he is more guarded, mistrusting of others as a result of his deployment experience.

Summary

Issue Identified	Level of Concern	Relationship to Deployment	Under Care
Military-Related PTSD Symptoms	Not a concern		
Civilian-Related PTSD Symptoms	Not a concern		
Depressive Symptoms	A minor concern	Somewhat related	Yes
Anxiety Symptoms	A minor concern	Somewhat related	Yes
Substance Use	A minor concern	Somewhat related	Yes
Post-Concussive Symptoms	Not a concern		
Other Physical Health Issues	Not a concern		
Family / Marital Problems	A minor concern	Somewhat related	Yes
Workplace Conflict	A major concern	Largely related	Yes
Suicidal Risk	A minor concern	Somewhat related	Yes
Other:			

Recommendations

Is a follow-up recommended? No

Comments

Mbr is already connected to MH services, both psychology and psychiatry. In meeting with him today, it does appear as though there has been some improvement in some of his symptoms over the past month with the treatment he is currently engaged in. Based on the results identified in the screening booklet, some of the above issues may have been viewed as a "major concern", however, based on his presentation today and feedback during the interview, writer has assessed them as being minor concerns, as mbr continues to experience symptoms but did identify feeling his mood is more stable. Follow up is not required in his case as he is already well-connected to the services/supports he requires, including psychiatry, psychology for individual psychotherapy, and couple counselling.

Signed by : Ms Jennifer L Grenier, SOCW, PT-MH, 2019-09-19 09:13:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Post-Deployment Assessment Document Date: 2019-09-19

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Scan-EPDS Questionnaire Booklet Document Date: 2019-09-19



DEMOGRAPHIC QUESTIONNAIRE

Surname	BARDYLA	Initials	RJ	Rank/Civ	MCpl
Service/NPF Number	T56588965	Cur Unit	CSOR	Age	37
Today's Date	20-08-2019	Operation	Tropical Dagger	Roto	
Date Returned from deployment		28-11-2018			

1. Branch of Service:
- ☒ 1 Land ☐ 2 Sea ☐ 3 Air ☐ 4 Civilian

2. Duty Status:
- ☒ 1 Regular ☐ 2 Reserves ☐ 3 Civilian

3. How many years and months, in total, have you been (were you) in any branch of the military, including the reserves?

Number of Years 09 Number of Months 07

4. Number of Previous Deployments (excluding most recent one); include domestic operations such as the Winnipeg flood, Ontario/Quebec ice storm, and the Swissair crash. (If you have not deployed before, please enter "00")

Number of previous deployments 01

5. How long was your most recent deployment?

☐ 1 Less than 2 months
☐ 2 2 to 5 months
☒ 3 6 to 8 months
☐ 4 9 months or more

6. Marital Status:

☒ 1 Married/living with partner ☐ 4 Separated
☐ 2 Divorced ☐ 5 Widowed
☐ 3 Single (never married)

7. How many dependent children do you have who reside with you on a regular basis?

Number of children 02

8. Did you have any injury or illness during this deployment?

☐ 1 Yes ☐ 2 No ☒ 3 Don't know or uncertain

9. If yes, have you filed a Declaration of Injury or illness during service in a Special Duty Area (CFMO 27-03)?

☐ 1 Yes ☐ 2 No ☒ 3 Don't know or uncertain

10. Sex

☒ 1 Male
☐ 2 Female

11. What language do you speak most often at home?

☒ 1 English
☐ 2 French
☐ 3 Other

12. What is the highest level of education that you have ever completed (excludes military trades training)?

☐ 1 High School or less
☒ 2 More than High School but less than University
☐ 3 University degree

13. What was your approximate total household income* for the previous tax year before income tax deduction? (please mark the appropriate box below—your best guess is just fine)

☐ 1 \$59K or less
☒ 2 \$60K to \$99K
☐ 3 \$100K or more
☐ 4 Don't know

*Household income is defined as any income received for all of the people living with you, including: wages and salaries; other income sources such as self-employment, unemployment insurance, worker's compensation, retirement income, interest and dividends on bonds, deposits, savings, and other investments.



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Scan-EPDS Questionnaire Booklet Document Date: 2019-09-19



National
Défense

PROTECTED B (when completed)

PROTÉGÉ B (une fois rempli) VERSION 6.0

Surname, Init.: Bardyla, RJ

Service Number: T56588965

THE SF-36® HEALTH SURVEY

Instructions for Completing the Questionnaire

Please answer every question. Some questions may look like others, but each one is different. Please take the time to read and answer each question carefully by checking the box that best represents your response.

EXAMPLE

This is for your review. Do not answer this question. The questionnaire begins with the section **Your Health in General** below.

For each question you will be asked to put an "X" the box in each line:

1. How strongly do you agree or disagree with each of the following statements?

	Strongly agree	Agree	Uncertain	Disagree	Strongly disagree
a) I enjoy listening to music.	☐	☒	☐	☐	☐
b) I enjoy reading magazines.	☒	☐	☐	☐	☐

Please begin answering the questions now.

Your Health in General

1. In general, would you say your health is:

Excellent	Very good	Good	Fair	Poor	
☐ 1	☐ 2	☒ 3	☐ 4	☐ 5	GH1

2. Compared to one year ago, how would you rate your health in general now?

Much better now than one year ago	Somewhat better now than one year ago	About the same as one year ago	Somewhat worse now than one year ago	Much worse now than one year ago	
☐ 1	☐ 2	☐ 3	☒ 4	☐ 5	HT



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3. The following items are about activities you might do during a typical day. Does your health now limit you in these activities? If so, how much?

	Yes, limited a lot	Yes, limited a little	No, not limited at all	
a) Vigorous activities, such as running, lifting heavy objects, participating in strenuous sports	☐ ₁	☒ ₂	☐ ₃	PF01
b) Moderate activities, such as moving a table, pushing a vacuum cleaner, bowling, or playing golf	☐ ₁	☐ ₂	☒ ₃	PF02
c) Lifting or carrying groceries	☐ ₁	☐ ₂	☒ ₃	PF03
d) Climbing several flights of stairs	☐ ₁	☐ ₂	☒ ₃	PF04
e) Climbing one flight of stairs	☐ ₁	☐ ₂	☒ ₃	PF05
f) Bending, kneeling, or stooping	☐ ₁	☒ ₂	☐ ₃	PF06
g) Walking more than a mile	☐ ₁	☐ ₂	☒ ₃	PF07
h) Walking several blocks	☐ ₁	☐ ₂	☒ ₃	PF08
i) Walking one block	☐ ₁	☐ ₂	☒ ₃	PF09
j) Bathing or dressing yourself	☐ ₁	☐ ₂	☒ ₃	PF10

4. During the past 4 weeks, have you had any of the following problems with your work or other regular daily activities as a result of your physical health?

	Yes	No	
a) Cut down on the amount of time you spent on work or other activities	☐ ₁	☒ ₂	RP1
b) Accomplished less than you would like	☐ ₁	☒ ₂	RP2
c) Were limited in the kind of work or other activities	☐ ₁	☒ ₂	RP3
d) Had difficulty performing the work or other activities (for example, it took extra effort)	☐ ₁	☒ ₂	RP4

5. During the past 4 weeks, have you had any of the following problems with your work or other regular daily activities as a result of any emotional problems (such as feeling depressed or anxious)?

	Yes	No	
a) Cut down on the amount of time you spent on work or other activities	☒ ₁	☐ ₂	RE1
b) Accomplished less than you would like	☒ ₁	☐ ₂	RE2
c) Did work or other activities less carefully than usual	☒ ₁	☐ ₂	RE3

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6. During the past 4 weeks, to what extent has your physical health or emotional problems interfered with your normal social activities with family, friends, neighbours, or groups?

Not at all Slightly Moderately Quite a bit Extremely
☐1 ☐2 ☐3 ☐4 ☒5 SF1

7. How much bodily pain have you had during the past 4 weeks?

None Very mild Mild Moderate Severe Very severe
☐1 ☒2 ☐3 ☐4 ☐5 ☐6 BP1

8. During the past 4 weeks, how much did pain interfere with your normal work (including both work outside the home and housework)?

Not at all A little bit Moderately Quite a bit Extremely
☐1 ☒2 ☐3 ☐4 ☐5 BP2

9. These questions are about how you feel and how things have been with you during the past 4 weeks. For each question, please give the one answer that comes closest to the way you have been feeling. How much of the time during the past 4 weeks...

	All of the time	Most of the time	A good bit of the time	Some of the time	A little of the time	None of the time	
a) did you feel full of pep?	☒ 1	☐ 2	☐ 3	☐ 4	☐ 5	☒ 6	VT1
b) have you been a very nervous person?	☒ 1	☐ 2	☐ 3	☐ 4	☐ 5	☒ 6	MH1
c) have you felt so down in the dumps nothing could cheer you up?	☒ 1	☐ 2	☐ 3	☐ 4	☐ 5	☒ 6	MH2
d) have you felt calm and peaceful?	☐ 1	☐ 2	☐ 3	☐ 4	☒ 5	☐ 6	MH3
e) did you have a lot of energy?	☒ 1	☐ 2	☐ 3	☐ 4	☐ 5	☒ 6	VT2
f) have you felt downhearted and blue?	☒ 1	☐ 2	☐ 3	☐ 4	☐ 5	☒ 6	MH4
g) did you feel worn out?	☒ 1	☐ 2	☐ 3	☐ 4	☐ 5	☒ 6	VT3
h) have you been a happy person?	☒ 1	☐ 2	☐ 3	☐ 4	☐ 5	☒ 6	MH5
i) did you feel tired?	☐ 1	☒ 2	☐ 3	☐ 4	☐ 5	☒ 6	VT4

10. During the past 4 weeks, how much of the time has your physical health or emotional problems interfered with your social activities (like visiting friends, relatives, etc.)?

All of the time Most of the time Some of the time A little of the time None of the time
☒1 ☐2 ☐3 ☐4 ☐5 SF2

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11. How TRUE or FALSE is each of the following statements for you?

	Definitely true	Mostly true	Don't know	Mostly false	Definitely false	
a) I seem to get sick a little easier than other people	☐ 1	☐ 2	☐ 3	☐ 4	☒ 5	GH2
b) I am as healthy as anybody I know	☐ 1	☐ 2	☒ 3	☐ 4	☐ 5	GH3
c) I expect my health to get worse	☐ 1	☐ 2	☒ 3	☐ 4	☐ 5	GH4
d) My health is excellent	☐ 1	☒ 2	☐ 3	☐ 4	☐ 5	GH5

Please turn the page and continue with the next questionnaire.



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Bardyla RJ
T56588965

PATIENT HEALTH QUESTIONNAIRE (PHQ)

This questionnaire is an important part of providing you with the best health care possible. Your answers will help in understanding problems that you may have. Please answer every question to the best of your ability unless you are requested to skip a question.

1. During the last 4 weeks, how much have you been bothered by any of the following problems?

	Not bothered	Bothered a little	Bothered a lot
a) Stomach pain	☐ 1	☒ 2	☐ 3
b) Back pain	☐ 1	☒ 2	☐ 3
c) Pain in your arms, legs, or joints (knees, hips, etc.)	☒ 1	☐ 2	☐ 3
d) Menstrual cramps or other problems with your periods	☒ 1	☐ 2	☐ 3
e) Pain or problems during sexual intercourse	☒ 1	☐ 2	☐ 3
f) Headaches	☐ 1	☒ 2	☐ 3
g) Chest pain	☒ 1	☐ 2	☐ 3
h) Dizziness	☐ 1	☒ 2	☐ 3
i) Fainting spells	☒ 1	☒ 2	☐ 3
j) Feeling your heart pound or race	☐ 1	☒ 2	☐ 3
k) Shortness of breath	☒ 1	☐ 2	☐ 3
l) Constipation, loose bowels, or diarrhoea	☐ 1	☒ 2	☐ 3
m) Nausea, gas, or indigestion	☒ 1	☐ 2	☐ 3
n) Problems thinking clearly	☐ 1	☐ 2	☒ 3
o) Making slips of the tongue while speaking	☐ 1	☐ 2	☒ 3
p) Problems with forgetfulness, like forgetting where you put things or forgetting appointments	☐ 1	☐ 2	☒ 3
q) Ringing in the ears	☐ 1	☐ 2	☒ 3
r) Balance problems	☐ 1	☒ 2	☐ 3

2. Over the last 2 weeks, how often have you been bothered by any of the following problems?

	Not at all	Several days	More than half the days	Nearly every day
a) Little interest or pleasure in doing things	☐ 1	☐ 2	☐ 3	☒ 4
b) Feeling down, depressed, or hopeless	☐ 1	☐ 2	☐ 3	☒ 4
c) Trouble falling or staying asleep, or sleeping too much	☐ 1	☐ 2	☒ 3	☐ 4
d) Feeling tired or having little energy	☐ 1	☐ 2	☒ 3	☐ 4
e) Poor appetite or overeating	☐ 1	☐ 2	☒ 3	☐ 4
f) Feeling bad about yourself, or that you are a failure, or have let yourself or your family down	☐ 1	☐ 2	☐ 3	☒ 4



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2. (Continued)... Over the last 2 weeks, how often have you been bothered by any of the following problems?

	Not at all	Several days	More than half the days	Nearly every day
g) Trouble concentrating on things, such as reading the newspaper or watching television	☐ 1	☐ 2	☒ 3	☐ 4
h) Moving or speaking so slowly that other people could have noticed. Or the opposite, being so fidgety and restless that you have been moving around a lot more than usual.	☒ 1	☐ 2	☐ 3	☐ 4
i) Thoughts that you would be better off dead, or of hurting yourself in some way	☐ 1	☐ 2	☒ 3	☐ 4

3. Questions about anxiety.

	NO	YES
a) In the <u>last 4 weeks</u> , have you had an anxiety attack—suddenly feeling fear or panic?	☐ 1	☒ 2
If you checked "NO," go to question 4.		
b) Has this ever happened to you before?	☐ 1	☒ 2
c) Do some of these attacks <u>come suddenly out of the blue</u> —that is, in situations where you don't expect to be nervous or uncomfortable?	☐ 1	☒ 2
d) Do these attacks bother you a lot or are you worried about having another attack?	☐ 1	☒ 2

4. Over the last 4 weeks, how often have you been bothered by the following problems?

	Not at all	Several days	More than half the days
Feeling nervous, anxious, on edge, or worrying a lot about different things	☐ 1	☐ 2	☒ 3
If you checked "Not at all," go to question 5.			
a) Feeling restless so that it is hard to sit still	☐ 1	☐ 2	☒ 3
b) Getting tired very easily	☐ 1	☐ 2	☒ 3
c) Muscle tension, aches, soreness	☐ 1	☐ 2	☒ 3
d) Trouble falling asleep or staying asleep	☐ 1	☒ 2	☐ 3
e) Trouble concentrating on things, such as reading a book or watching television	☐ 1	☐ 2	☒ 3
f) Becoming easily annoyed or irritated	☐ 1	☐ 2	☒ 3

5. In the last 4 weeks, how much have you been bothered by any of the following problems?

	Not bothered	Bothered a little	Bothered a lot
a) Worrying about your health	☐ 1	☐ 2	☒ 3
b) Your weight or how you look	☐ 1	☐ 2	☒ 3
c) Little or no sexual desire or pleasure during sex	☐ 1	☐ 2	☒ 3
d) Difficulties with husband/wife, partner/lover, or boyfriend/girlfriend	☐ 1	☐ 2	☒ 3
e) The stress of having to take care of children, parents, or other family members	☐ 1	☐ 2	☒ 3



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5. (Continued)... In the last 4 weeks, how much have you been bothered by any of the following problems?

	Not bothered	Bothered a little	Bothered a lot
f) Stress at work outside of the home or at school	☐ 1	☐ 2	☒ 3
g) Financial problems or worries	☐ 1	☒ 2	☐ 3
h) Having no one to turn to when you have a problem	☐ 1	☐ 2	☒ 3
i) Something bad that happened <u>recently</u>	☐ 1	☐ 2	☒ 3
j) Thinking or dreaming about something terrible that happened to you in the past—like your house being destroyed, a severe accident, being hit or assaulted, or forced to commit a sexual act	☐ 1	☒ 2	☐ 3

6. In the past year, have you been hit, slapped, kicked, or otherwise physically hurt by someone, or has anyone forced you to have an unwanted sexual act?

NO YES
☐1 ☒2

6a. If yes, please describe: while on Ex a peer started a fist fight with me

7. What is the most stressful thing in your life right now? Work

	NO	YES
8. Are you taking any medication for anxiety, depression, or stress?	☐ 1	☒ 2
9. Have you ever <u>seen</u> , or talked to on the telephone, a health professional about your emotional or mental health? [Do NOT include routine pre- or post-deployment screening.]	☐ 1	☒ 2
10. Are you <u>currently seeing</u> a health professional about your emotional or mental health?	☐ 1	☒ 2

Please turn the page and continue with the next questionnaire.



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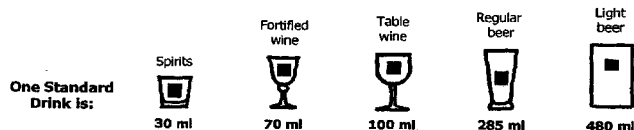
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AUDIT

The following questions relate to alcohol use. Please check the box that best corresponds to your answer.
PLEASE ANSWER ALL QUESTIONS—EVEN IF YOU DO NOT CURRENTLY DRINK ALCOHOL



1. How often do you have a drink containing alcohol?

never ☐1 monthly or less ☒2 2 to 4 times a month ☐3 2 to 3 times a week ☐4 4 or more times a week ☐5

2. How many drinks containing alcohol do you have on a typical day when you are drinking?

none ☐0 1 or 2 ☐1 3 or 4 ☒2 5 or 6 ☐3 7 to 9 ☐4 10 or more ☐5

	Never	Less than monthly	Monthly	Weekly	Daily or almost daily
3. How often do you have six or more drinks on one occasion?	☒ 1	☒ 2	☐ 3	☐ 4	☐ 5
4. How often during the last year have you found that you were not able to stop drinking once you had started?	☐ 1	☐ 2	☐ 3	☐ 4	☒ 5
5. How often during the last year have you failed to do what was normally expected of you because of drinking?	☐ 1	☐ 2	☐ 3	☐ 4	☒ 5
6. How often during the last year have you needed a drink in the morning to get yourself going after a heavy drinking session?	☒ 1	☐ 2	☐ 3	☐ 4	☐ 5
7. How often during the last year have you had a feeling of guilt or remorse after drinking?	☒ 1	☐ 2	☐ 3	☐ 4	☐ 5
8. How often during the past year have you been unable to remember what happened the night before because you had been drinking?	☐ 1	☐ 2	☐ 3	☒ 4	☐ 5

9. Have you or someone else been injured as a result of your drinking?

No ☐1 Yes, but not during the last year ☒2 Yes, during the last year ☐3

10. Has a relative, friend, a doctor, or other health professional been concerned about your drinking or suggested you should cut down?

No ☐1 Yes, but not during the last year ☐2 Yes, during the last year ☒3

Please turn the page and continue with the next questionnaire.

AUDIT © 1992, World Health Organization.

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PCL-C¹

Below is a list of problems and complaints that service members and veterans sometimes have in response to stressful life experiences. Please read each one carefully, put an "X" in the box to indicate how much you have been bothered by that problem in the last month.

	Not at all	A little bit	Moderately	Quite a bit	Extremely
1. Repeated, disturbing memories, thoughts, or images of a stressful experience from the past?	☐ ₁	☐ ₂	☒ ₃	☐ ₄	☐ ₅
2. Repeated, disturbing dreams of a stressful experience from the past?	☐ ₁	☐ ₂	☒ ₃	☐ ₄	☐ ₅
3. Suddenly acting or feeling as if a stressful experience were happening again (as if you were reliving it)?	☒ ₁	☐ ₂	☐ ₃	☐ ₄	☐ ₅
4. Feeling very upset when something reminded you of a stressful experience from the past?	☐ ₁	☐ ₂	☐ ₃	☒ ₄	☐ ₅
5. Having physical reactions (e.g., heart pounding, trouble breathing, or sweating) when something reminded you of a stressful experience from the past?	☒ ₁	☐ ₂	☐ ₃	☐ ₄	☐ ₅
6. Avoid thinking about or talking about a stressful experience from the past or avoid having feelings related to it?	☐ ₁	☐ ₂	☒ ₃	☐ ₄	☐ ₅
7. Avoid activities or situations because they remind you of a stressful experience from the past?	☐ ₁	☐ ₂	☐ ₃	☐ ₄	☒ ₅
8. Trouble remembering important parts of a stressful experience from the past?	☐ ₁	☐ ₂	☐ ₃	☒ ₄	☐ ₅
9. Loss of interest in things that you used to enjoy?	☐ ₁	☐ ₂	☐ ₃	☒ ₄	☐ ₅
10. Feeling distant or cut off from other people?	☐ ₁	☐ ₂	☐ ₃	☐ ₄	☒ ₅
11. Feeling emotionally numb or being unable to have loving feelings for those close to you?	☐ ₁	☐ ₂	☐ ₃	☒ ₄	☐ ₅
12. Feeling as if your future will somehow be cut short?	☐ ₁	☐ ₂	☐ ₃	☒ ₄	☐ ₅
13. Trouble falling or staying asleep?	☐ ₁	☒ ₂	☐ ₃	☐ ₄	☐ ₅
14. Feeling irritable or having angry outbursts?	☐ ₁	☐ ₂	☐ ₃	☐ ₄	☒ ₅
15. Having difficulty concentrating?	☐ ₁	☐ ₂	☐ ₃	☐ ₄	☒ ₅
16. Being "super alert" or watchful or on guard?	☐ ₁	☒ ₂	☐ ₃	☐ ₄	☐ ₅
17. Feeling jumpy or easily startled?	☐ ₁	☒ ₂	☐ ₃	☐ ₄	☐ ₅

18. Did the stressful experience(s) in the past that you referred to above occur ...(check all that apply if you were referring to more than one stressful experience):

- a) ☐ As part of your private life (meaning prior to military service or while off duty).
- b) ☒ As part of your military service while deployed
- c) ☐ As part of your military service while on duty and not deployed.
- d) ☐ Not applicable (no stressful experiences).

¹ PCL-C © 1991, National Center for PTSD (USA)



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Surname, Init.: Bardyla RJ
Service Number: T56 588965

DEPLOYMENT EXPERIENCES²

During your most recent deployment, did you experience:	NO	YES
1. Being attacked or ambushed	☒ 1	☐ 2
2. Receiving small arms fire	☒ 1	☐ 2
3. Seeing dead bodies or human remains	☐ 1	☒ 2
4. Handling or uncovering human remains	☒ 1	☐ 2
5. Witnessing an accident which resulted in serious injury or death	☒ 1	☐ 2
6. Seeing dead or seriously injured Canadians	☒ 1	☐ 2
7. Knowing someone seriously injured or killed	☒ 1	☐ 2
8. Improvised explosive device (IED)/booby trap exploded near you	☒ 1	☐ 2
9. Working in areas that were mined or had IEDs	☒ 1	☐ 2
10. Having hostile reactions from civilians	☐ 1	☒ 2
11. Being in threatening situations where you were unable to respond because of rules of engagement	☒ 1	☐ 2
12. Shooting or directing fire at the enemy	☒ 1	☐ 2
13. Calling in fire on the enemy	☒ 1	☐ 2
14. Engaging in hand-to-hand combat	☒ 1	☐ 2
15. Clearing/searching homes or buildings	☒ 1	☐ 2
16. Clearing/searching caves or bunkers	☒ 1	☐ 2
17. Being wounded/injured	☒ 1	☐ 2
18. Seeing ill/injured women or children who you were unable to help	☒ 1	☐ 2
19. Receiving incoming artillery, rocket, or mortar fire	☒ 1	☐ 2
20. Feeling directly responsible for the death of an enemy combatant	☒ 1	☐ 2
21. Feeling directly responsible for the death of a non-combatant	☒ 1	☐ 2
22. Witnessing a friendly fire incident	☒ 1	☐ 2
23. Feeling responsible for the death of Canadian or ally personnel	☒ 1	☐ 2
24. Having a member of your own unit become a casualty	☒ 1	☐ 2
25. Had a close call, was shot or hit but protective gear saved you	☒ 1	☐ 2
26. Had a buddy shot or hit who was near you	☒ 1	☐ 2
27. Having difficulty distinguishing between combatants and noncombatants	☒ 1	☐ 2
28. Participating in IED/mine clearing	☒ 1	☐ 2
29. Sniper fire	☒ 1	☐ 2
30. Seeing a unit member blown up or burned alive	☒ 1	☐ 2

Please turn the page and continue with the next questionnaire.

² Adapted from the US Army's Walter Reed Army Institute for Research Combat Exposure Scale
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Service Number: T56588965

INJURIES³

1. Did you have any injury(ies) during your deployment from any of the following?

(check all that apply):

☐	A. Fragment
☐	B. Bullet
☐	C. Vehicular (any type of vehicle, including airplane)
☐	D. Fall
☐	E. Blast (Improvised Explosive Device, RPG, Land mine, Grenade, etc.)
☐	F. Other specify: _____

2. Did any injury received while you were deployed result in any of the following?

(check all that apply):

☐	A. Being dazed, confused or "seeing stars"
☐	B. Not remembering the injury
☐	C. Losing consciousness (knocked out) for less than a minute
☐	D. Losing consciousness for 1-20 minutes
☐	E. Losing consciousness for longer than 20 minutes
☐	F. Having any symptoms of concussion afterward (such as headache, dizziness, irritability, etc.)
☐	G. Head Injury
☒	H. None of the above

	NO	YES
3. During this deployment, were you evaluated by a medical professional for a TBI (Traumatic Brain Injury) or concussion?	☒ 1	☐ 2

END OF QUESTIONNAIRES

THANK YOU!

Please return this booklet to the screening staff; they will summarize your responses for the staff member who is doing your post-deployment screening.

³ Adapted from US DVBIC 3 question screening tool.

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Scan-EPDS Questionnaire Booklet Document Date: 2019-09-19

Signed by : Ms Wanda C Ziebarth, CLERK, PT-MH, 2019-09-19 09:23:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Scan-Enhanced Post Deployment Report Document Date: 2019-09-19

PROTECTED B (once completed - PROTEGE B (une fois rempli))

Bardyla MCpl RJ
T56 588 965

Date of Survey: 20-Aug-2019
TRACKING NUMBER: 225051

Enhanced Post-deployment Screening: Survey Report

Demographics

Military Characteristics	
Rank	MCpl
CURRENT UNIT CSOR	
Operation	MISSING
Rotation	MISSING
Deployment length (months)	6 - 8 mo.
Time since return (months)	8.7
Environment	Land
Component	Regular Forces
Years in military	9.6
Num. of previous deployments	1

Personal Characteristics	
Age (years):	37
Sex	Male
Marital Status	Married/living with partner
Number of children residing w/ member:	2
Language spoken at home:	English
Educational attainment:	More than high school but less than university



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
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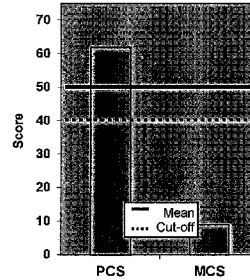
Date of Survey:
TRACKING NUMBER:

20-Aug-2019

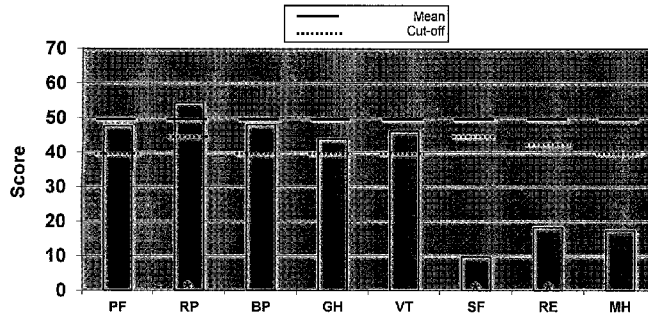
SF-36 Interpretation Process:

1. First, look at the Component Summary Scores on the upper right. The Physical and Mental Component Summary Scores (PCS, MCS) reflect overall physical and mental health status, respectively. These are also norm-based scores with a mean of about 50 and a standard deviation of about 10, with the reference population being more than 30,000 CF members completing post-deployment screening since 2002. Lower scores reflect poorer health. The solid line indicates the population mean score, and the line indicates 1 standard deviation below the mean, which corresponds to about the 15th percentile.
2. If both the PCS and MCS scores are above the lower dotted line, it is very unlikely that the individual's physical or mental health are seriously impaired; if a mental or physical disorder were present, it would likely be a mild one.
3. Next, look at the graph at the bottom left to see which of the component scales may be contributing to impaired health. These are also norm-based scores with a mean of about 50 and a standard deviation of about 10. The solid lines again denote the population mean. The dotted line indicates the score below which further questioning is recommended. For most scales, the cut-off score is 1 standard deviation below the mean (again, approximately the 15th percentile). However, for scales reflecting functional status (RP, SF, and RE—marked with an *), the cut-off score is set somewhat higher, so that those with ANY impairment in role functioning or more than minimal impairment in social functioning will be flagged.
4. For the scales that tend to be most strongly influenced by recent deployment (GH, VT, SF, and MH), investigation of scores that are just marginally above the cut-off may be helpful, particularly if the person has returned from a less demanding deployment (where more subtle health impacts are expected).

SF-36 Component Summary Scores



SF-36 Norm-based Scores



Physical Functioning
Role-Physical
Bodily Pain
General Health
Vitality
Social Functioning
Role-Emotional
Mental Health

Physical Component Summary
Mental Component Summary

Note: If any scale score = 0, this may indicate that less than 1/2 of the questions on that scale were answered, in which case the answer is not valid. Alternatively, if 1/2 or more of the questions on that scale were answered, a scale score = 0 reflects a true low score.



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Scan-Enhanced Post Deployment Report Document Date: 2019-09-19

PROTECTED B (once completed - PROTEGE B (une fois rempli))

Bardyla MCpl RJ
T56 588 965

Date of Survey:
TRACKING NUMBER:

20-Aug-2019
225051

POTENTIAL PROBLEMS	SCREENING TEST RESULTS	POSITIVE OR MISSING RESPONSES TO EXPLORE
PHYSICAL HEALTH		
Perceived health status	GOOD	
Change in health (past year)	SOMEWHAT WORSE	SF-36 #2
Global physical health (SF-36 PCS)	62 (LOW RISK)	
Somatic symptoms (PHQ)	10 (HIGH RISK)	BOTHERED A LOT: #1n [problems thinking clearly] #1o [slips of the tongue] #1p [forgetfulness] #1q [ringing in the ears] #2c [sleep problems] #2d [fatigue] MISSING / BOTHERED A LITTLE: #1a [stomach pain] #1b [back pain] #1f [headaches] #1h [dizziness] #1j [heart pounding/racing] #1l [bowel probs.] #1r [balance probs.]
Concussion (PHQ, PCL-C, OVBIG)	NONE	
MENTAL HEALTH		
Global mental health (SF-36 MCS)	9 (HIGH RISK)	#6, #10 [Social Functioning] #5a - c [Role-Emotional] #9b, c, d, f, h [Mental Health]
Mental health care (PHQ)	CURRENT CARE	#8 [taking psych meds] #9 [mental health care ever] #10 [mental health care currently]
Depression (PHQ)	PHQ-9 SCORE = 19 (MODERATELY SEVERE DEPRESSION)	MORE THAN HALF THE DAYS / NEARLY EVERY DAY: #2a [anhedonia] #2b [depressed mood] #2c [sleep problems] #2d [fatigue] #2e [appetite changes] #2f [guilt] #2g [trouble concentrating] #2i [suicidality] MISSING / SEVERAL DAYS: NONE
Suicidality (PHQ)	MORE THAN HALF THE DAYS	#2i [suicidality]
Panic attacks (PHQ)	YES (PANIC DISORDER LIKELY)	#3a [panic attack in last 4 weeks] #3b [attack(s) in past] #3c [attacks(s) "out of the blue"] #3d [bothered/worried about attack(s)]
Generalized anxiety (PHQ)	PHQ-7 SCORE = 13 (SEVERE ANXIETY)	MORE THAN HALF THE DAYS: #4 [nervousness, anxiety, on edge, or worry] #4a [restlessness] #4b [fatigability] #4c [muscle tension or pain] #4e [trouble concentrating] #4f [irritability] MISSING / SEVERAL DAYS: #4d [sleep difficulties]
Stressors (PHQ)	STRESS INDEX = 18 (HIGH RISK)	ABUSE IN LAST YEAR: YES: 'while on EX a peer started a fist fight with me' MOST STRESSFUL THING RIGHT NOW: work BOTHERED A LOT: #5a [health worries] #5b [weight/appearance] #5c [decreased sexual desire] #5d [difficulties with partner] #5e [caretaker stress] #5f [stress at work/school] #5h [no one to turn to] #5i [recent bad event] MISSING / BOTHERED A LITTLE: #5g [financial worries] #5j [traumatic recollection]
High-risk drinking (AUDIT)	AUDIT SCORE = 20 (HIGH RISK)	#2 [usually drinks 3 - 4 drinks] #3 [binge drinking less than monthly] #4 [loss of control] #5 [failed to do what was expected] #8 [blackouts] #9 [injury to self/others] #10 [someone concerned about drinking or told to cut back]
Deployment experiences	YES	PERSONAL THREAT/HARM: #10 [hostile civilians] HARMING OTHERS: NONE WITNESSING THREAT/HARM: #3 [seeing bodies/remains]
PTSD (PCL-C)	PCL-C SCORE = 57 (HIGH RISK FOR PTSD)	MODERATELY TO EXTREMELY: #1 [memories/thoughts/images] #2 [nightmares] #4 [upset when reminded] #6 [avoiding thinking/talking] #7 [avoiding activities/situations] #8 [trouble remembering] #9 [anhedonia] #10 [distant/cut-off] #11 [emotionally numb] #12 [future cut short] #14 [irritable/angry] MISSING / A LITTLE BIT: #13 [insomnia] #16 super alert/watchful/on guard] #17 [jumpy/easily startled]

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END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Scan-Enhanced Post Deployment Report Document Date: 2019-09-19

Signed by : Ms Wanda C Ziebarth, CLERK, PT-MH, 2019-09-19 09:20:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-09-11

Reset - Réinitialiser

Arrival Status - Ponctualité Was on time - Ponctuel ☒	Session Took Place - Lieu de la session Office - Bureau ☒
Note :	Note :
Service Provided - Service offert Couple / Family - Couple / famille ☒	Session Number - Numéro de la session 1
Treatment Approach / Intervention - Approche de traitement / Intervention	
Main - Principale : Other	
Additional - Supplémentaire :	
Additional - Supplémentaire :	
Note : Gottman: feedback	
Narrative - Narration	
Mbr and spouse attend session awaiting feedback on results of the survey and on direction of care Mbr and spouse report no changes in their way of being; spouse states they never fight, Mbr adds just being their normal self. Conversation opens then to sharing feedback on completing the survey as well as review of results that are pertinent in direction of therapy. Both partners share not wanting separation but also knowing things between them are not working for either partner. He reports being scared stiff of outcome of couple therapy for the simple reason of identifying all that is wrong with him in the relationship. She reports wanting to know what to do to make it better. Both share difficulty dealing with stress and negativity as they admit preferring to avoid issues . Feedback provided with good effect and a good discussion on direction of care; both agree needing to review negative pattern and learn their attachment style specifically when they communicate; he shares almost all the time seeing negative when she purports giving a compliment and she shares in her efforts to keeping it positive she is often met with negativity.	
Current Status - Statut actuel Unchanged - Inchangé ☒	Outcome Measures - Mesures d'impacts
	OQ - 45 :
	Other - Autre :



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-09-11

Clinical Impression - Impression clinique

Both share having a good session; both decide to return despite the topic to address in the negative pattern
Mood/affect: he shows fear and admits being scared at what to expect from couple therapy and only in last few minutes when observations as to his level of comfort in addressing issues is shared does he slightly calm; she offers encouragement and openness to be there with him however not knowing herself what works and not
He admits leaving it up to therapy to help him; reassurance offers to allow being apprehensive of the process but also the need for his participation.
Thought process coherent and however seemingly passive in their approach to couple therapy, they both report wanting it straight and to the point
Thought content to overcome the negativity they feel and allow the therapeutic process

Clinical Impression Classification - Classification d'impression clinique

1: Relationship distress with spouse or partner

2:

3:

Note :

Recommendations / Plan - Recommandations / Plan

Couple therapy

- review motivation and willingness to attend
- discuss issues for approx 10 minutes
- outline negative pattern as per their narrative
- explore ways they already show improvement

Patient Instructions - Instructions au patient

Both partners are invited to be kind toward one another as they address issues and to keep conversations light and focus on fun/playfulness to help rekindle passion/romance; both verbalize agreement

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2019-09-12 09:57:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Session Note Document Date: 2019-09-11

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Mental Health - Couple / Family Assessment Document Date: 2019-08-27

Reset

<u>Date of Referral</u> 2019-07-17		<u>Referral Source</u> Other <u>Note</u> : Psychiatrist																																	
<u>Primary Care Clinician or CDU</u>																																			
<u>Dates of Assessment</u> 1 : 2019-08-27 2 : 2019-09-18 3 : 2019-09-24		<u>Partner / Spouse</u> <u>Name</u> : Jan Bardyla <u>DOB</u> : <u>SN or Record #</u> : <u>PCC (if applicable)</u> :																																	
<u>Children or Other Dependents</u> ☒ Yes ☐ No																																			
<table border="1"><thead><tr><th>Name</th><th>DOB</th><th>Comments</th></tr></thead><tbody><tr><td>1 : Liam</td><td></td><td>M - 15</td></tr><tr><td>2 : Logan</td><td></td><td>M - 12</td></tr><tr><td>3 :</td><td></td><td></td></tr><tr><td>4 :</td><td></td><td></td></tr><tr><td>5 :</td><td></td><td></td></tr><tr><td>6 :</td><td></td><td></td></tr><tr><td>7 :</td><td></td><td></td></tr><tr><td>8 :</td><td></td><td></td></tr><tr><td>9 :</td><td></td><td></td></tr><tr><td>10 :</td><td></td><td></td></tr></tbody></table> <u>Note</u> :			Name	DOB	Comments	1 : Liam		M - 15	2 : Logan		M - 12	3 :			4 :			5 :			6 :			7 :			8 :			9 :			10 :		
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<u>Informed Consent</u> ☒ Yes The purpose of this clinical assessment was discussed. The client was given an opportunity to ask questions and agreed to participate. Additionally, the assessment process, involving a semi-structured clinical interview, the completion of several psychological inventories as indicated, and a review of the health record was discussed with the client. At the outset of the assessment, confidentiality, including the limits inherent, was explained and the MH Svcs information sheet was provided; the client expressed an understanding. It was explained that the report based on the findings of the assessment would be generated and forwarded to the client's Primary Care Clinician and placed on their health record.																																			
<u>Presenting Problem</u> Mbr and spouse request help with growing distance within the relationship to the point feeling like they are living as roommates rather than spouses, they say																																			
<u>Areas of Concern in Family / Relationship</u> <table border="0"><tr><td>☐ Addiction</td><td>☐ Infidelity</td><td>☐ Parenting</td></tr><tr><td>☐ Child Behaviour</td><td>☐ Legal or Disciplinary Concerns</td><td>☐ Perspective about division of labour</td></tr><tr><td>☒ Communication</td><td>☐ Mental Health Diagnosis in CAF member</td><td>☒ Relationship Intimacy</td></tr><tr><td>☐ Conflict Resolution</td><td>☐ Mental Health Diagnosis in Partner</td><td>☐ Sexual Relationship</td></tr><tr><td>☐ Financial</td><td>☐ Operational Stress Injury</td><td>☐ Spousal Abuse or Family Violence</td></tr><tr><td></td><td></td><td>☐ Other</td></tr></table> <u>Note</u> : <u>History of Presenting Problem</u>			☐ Addiction	☐ Infidelity	☐ Parenting	☐ Child Behaviour	☐ Legal or Disciplinary Concerns	☐ Perspective about division of labour	☒ Communication	☐ Mental Health Diagnosis in CAF member	☒ Relationship Intimacy	☐ Conflict Resolution	☐ Mental Health Diagnosis in Partner	☐ Sexual Relationship	☐ Financial	☐ Operational Stress Injury	☐ Spousal Abuse or Family Violence			☐ Other															
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Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Mental Health - Couple / Family Assessment Document Date: 2019-08-27

Mbr cannot remember happy moments in the relationship lately being too busy and almost always away or coming home when everyone is asleep, he says. Spouse shares not having a date night for the past 3 years. She withdraws to cope with his outbursts when he is home and he complains when she shows no emotions, both describe. They have been living together while leading separate lives, both share. Both admit getting caught up in each other's negative emotions and then to withdraw on their own to deal with it.

Contributing and Protective Factors

Mbr reports feeling betrayed not valued and admits to accumulation of stress only to explode in anger.

24 Sept: Mbr reports doing better with self-regulation of angry moments; working on it with DBT

She reports when having difficulty talking with him she retreats to herself and withdraws from the relationship.

Both deny abusive behavior toward one another.

Clients' Goals and Expectations

Both partners share wanting the relationship to improve as both feel unlike themselves and not liking the person they seem to have become over the years.

18 Sept: Spouse reports missing the closeness they have on and off in their relationship

24 Sept: Mbr reports desired outcome in therapy to find peace within himself whatever that may be.

Significant Family Special Needs, Medical or Mental Health Issues

Mbr attends multiple appointments with psychiatrist, psychologist and addictions counsellor and now couple therapy.

2014 - MDD, ADD

2019 - ADHD, OCD, BPD, Avoidant personality disorder

24 Sept: Mbr reports seeing traits of personality disorder and OCD but not full diagnosis as dysfunctional in his life. He shares continuing with individual therapy as it is helping rediscover himself

Past Mental Health / Psychosocial Services (within the family)

Spouse reports being followed and treated for anxiety/panic episodes

Relationship History

Mbr and spouse recount meeting 17 years ago. Mbr shares not thinking of a longterm relationship; he was ready to leave 3 months into the relationship when she was pregnant, he says. Mbr states being very competitive and when his buddies talked about her, he wanted to be the one to get her, he confesses. Both admit attraction of lust at the beginning. Within their first year and during her first pregnancy they split for one year, she recounts. They reconciled and describe the companionship that developed. However, he shares period of infidelity after their second child, they separated for approx a few months, he says. Both share missing each other when apart. He states noticing in the past eight years a change in her attitude toward him in terms of 'growing a backbone' and appreciating her more. Although he states wanting to work on the relationship, he also feels he does not care anymore, he admits. They live in their home with a menagerie of animals: 4 dogs, 2 cats and a lizard, and their 2 children. They report being in good financial standing.

Past and Present Safety Concerns

Mbr shares at age 12 taking on overdose of tylenol and hospitalized. He shares thinking more along the lines of disappearing from society rather than dying. Spouse denies ever having thoughts of suicide in harming self or others.

Past and Present Substance Use and / or Behavioural Addiction

Mbr reports being diagnosed with Cannabis and Alcohol use Disorder and both moderate; he believes having an addictive personality. He shares also able to be at a video game from when he wakes up on weekends until when he goes to bed and only if he decides to go to bed; he believes also having an addiction to video games.

Drinking pattern for both partners at this time is within their control, they both attest; occasional and social with family and friends, they describe. Both partners attest not having a problem at this time; Mbr continues to be followed by BAC, he assures.

Clinical Impression

Very genuine and candid couple with a history of attachment issues/injuries. During the interview when he was describing their first few years after meeting, she sat very quietly without reaction as if it was normal for them to leave out any affection in words as well as in gestures toward one another; they also sat as far apart as possible on the sofa. They both seek counselling but from very different perspective.

18 Sept: Spouse mentions doing much better together since first interview and she reports feeling closer than identified earlier; she shares believing in her husband.

24 Sept: Mbr shows linear thinking process; readiness, willingness and motivation to attend couple therapy and



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Mental Health - Couple / Family Assessment Document Date: 2019-08-27

putting in the required work, he says. He also shows candid and genuine self-disclosure that makes for open and frank conversation

Clinical Impression Classification

Administrative

- ☐ Compassionate request / CCM
- ☐ Need for information / resources
- ☐ OUTCAN - Isolated posting
- ☐ Release / retirement
- ☐ Other (Administrative)

Education

- ☐ Schooling unavailable / unattainable
- ☐ Underachievement in school
- ☐ Failed school examination
- ☐ Education maladjustment and discord with teachers and classmates
- ☐ Other (Education)

Legal or Disciplinary

- ☐ Victim of sexual harassment at work
- ☐ Victim of sexual harassment (outside of work)
- ☐ Victim of sexual crime at work
- ☐ Victim of sexual crime (outside of work)
- ☐ Victim of harassment at work
- ☐ Victim of harassment (outside of work)
- ☐ Victim of crime at work
- ☐ Victim of crime (outside of work)
- ☐ Target of (perceived) adverse discrimination or persecution
- ☐ Criminal prosecution
- ☐ Convicted in civil / criminal proceedings without imprisonment

Addictions / Compulsive Behaviors

- ☐ Alcohol
- ☐ Drugs illicit
- ☐ Drugs prescribed
- ☐ Tobacco
- ☐ Problematic eating behaviours
- ☐ Gambling / betting / VLT
- ☐ Sexual
- ☐ Video games
- ☐ Internet / Social Media
- ☐ Other (Addictions / Compulsive Behaviors)

Family Circumstances

- ☐ Absence of family member due to deployment or course
- ☐ Stress due to return of deployed member (reintegration issues)
- ☐ Stressful event affecting family or household
- ☐ Death of a family member / grief
- ☐ Alcohol use of a family member
- ☐ Substance use of a family member
- ☐ Other addiction of a family member
- ☐ Child abuse / neglect
- ☐ Domestic violence
- ☐ Parenting issues
- ☒ Relationship distress with spouse or partner
- ☐ Disruption of family by separation or divorce
- ☐ Relationship with in-laws
- ☐ Dependent needing care at home
- ☐ Other (Family Circumstances)

Occupational

- ☐ Discord with supervisor or colleagues
- ☐ Lack of job satisfaction
- ☐ Stressful work schedule
- ☐ Frequent postings / change job
- ☐ Frequent deployments
- ☐ Exposure to disaster, war and other hostilities
- ☐ First responder / stressful event
- ☐ Other (Occupational)

Childhood / Upbringing

- ☐ Inadequate parental supervision
- ☐ Parental overprotection
- ☐ Estrangement from one parent
- ☐ Removal from home during childhood (institution or foster care)
- ☐ Parent-child conflict in childhood
- ☐ Neglect in childhood
- ☐ Physical abuse in childhood
- ☐ Psychological abuse in childhood
- ☐ Sexual abuse in childhood
- ☐ Other (Childhood / Upbringing)

Housing and Economic Circumstances

- ☐ Discord with neighbours, lodgers or landlord
- ☐ Inadequate housing
- ☐ Low income
- ☐ Extreme poverty
- ☐ Financial hardship due to debt
- ☐ Other (Housing and Economic Circumstances)

Personal

- ☐ Adjustment to stress
- ☐ Anger
- ☐ Anxiety / stress symptoms
- ☐ Depressive symptoms
- ☐ Symptoms of post-traumatic stress
- ☐ Sleep Related Issues
- ☐ Other MH symptoms
- ☐ Chronic pain
- ☐ Phase of life problem
- ☐ Spiritual / religious
- ☐ Acculturation difficulty
- ☐ Sexual issues
- ☐ Sexual orientation / identity
- ☐ Unwanted pregnancy
- ☐ Other (Personal)



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Mental Health - Couple / Family Assessment Document Date: 2019-08-27

- ☐ Imprisonment / incarceration
- ☐ Military recorded warning
- ☐ Military counseling and probation
- ☐ Military court martial
- ☐ Other (Legal or Disciplinary)

Note :

Individual Session (if applicable)

See Report Dated:

2019-04-15

Partner's Individual Session (if applicable)

See Report Dated:

Recommendations / Plan

Mbr and spouse agree to complete Gottman Relationship Survey

- Review negative pattern in their communication style
- Discuss attachment style within injuries sustained as a child for Mbr and the link to issues developed within their relationship
- Explore healthy ways to show and receive affection, appreciation and repair attempts.

Patient Instructions

Mbr and spouse are invited to observe their emotions and reactions with one another specifically when they argue and to prepare to talk about their way of being in communicating their needs.

Both agree; spouse books her individual appointment for 1:1; they book their next couple session in 2 weeks.

Self-care practices: Mbr is aware of potential information overload with the number of appointments in the next month with caution to pay attention to self-care needs; Mbr agrees.

24 Sept: Mbr is encouraged to eradicate blaming from his thought process; although difficult, he says understanding the usefulness in the coupleship. He is also provided with values clarification exercise to help with identifying and describing emotions, thoughts and behavior. Link to DBT workbook is provided with invite to return for 1:1 as needed to further discuss mindfulness; Mbr agrees and books another 1:1 in 2 weeks

Signed by : Ms Shirley E Gorham, RN, PT-MH, 2019-09-24 10:06:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Mental Health - Couple / Family Assessment Document Date: 2019-08-27

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Scan-Consent Document Date: 2019-08-27

PROTECTED B (When completed)

ANNEX A to MH Svcs Informed Consent
Interim Guidance R2000 Doc 17.13.2

Statement of Patient Understanding -
Mental Health or Psychosocial Assessment and Treatment

- Please review the following information and discuss any questions or concerns you may have with your health care provider.
- Confidentiality.** The Mental Health Department has a strict policy about confidentiality. In accordance with the code of ethics that governs the professional conduct of health care providers involved in your care, the information shared by you will not be given to anyone without your permission unless required or authorized by law or CF policy as set out in CF H Svcs Gp Instruction 5020-20 and CANFORGEN 039/08.
- Instruction 5020-20 states, in part, "In accordance with the *Privacy Act*, and subject to legislated exceptions, no specific diagnosis, course of treatment, therapy or care may be disclosed to, or discussed with, the CO or any non-health services personnel, without the member's consent ... Any medical employment limitations that have been assigned to a CF member because of a health or psychosocial condition, as well as the prognosis of that condition (where possible), will be fully described and explained to the member's CO by the member's health care provider."
- Results of assessment and treatment will be discussed with you and recorded in your psychosocial file or CF medical record. CF Mental Health Services function on an interdisciplinary care service model. Our team is composed of psychiatrists, psychologists, social workers, mental health nurses and addiction counsellors. At Operational Trauma Stress Support Centres (OTSSC), the core health care team also includes chaplains. At OTSSCs, representatives from the Operational Stress Injury Social Support (OSISS) program function as non-health care supports. The results of your assessment and treatment will be discussed with members of the core mental health care team in order to best serve your needs. Your medical information may also be shared with members of the extended health care team with your written consent.
- Other situations obliging CF Health Services to disclose information without your consent may include:
 - Disclosure of physical, sexual or mental neglect or harm to a minor.
 - Disclosure of risk to your own safety or safety of others.
 - Disclosure of sexual abuse or harassment by a regulated health care professional.
 - If testimony of your health care provider or disclosure of your treatment/medical records is ordered by Board of Inquiry, Police investigation, Court of Law, or otherwise authorized by law.
- Treatment.** Professional staff are expected to actively involve you in decisions about your care. Similarly, you have the responsibility to actively engage in carrying out those decisions, including attendance at all appointments made related to your care.
- Safety.** The Clinic is committed to ensuring a safe and respectful environment for all patients and staff. Good conduct is expected at all times in military health care settings. While expressions of personal distress may occur during therapeutic interventions, these should not extend to threatening, disrespectful or otherwise unacceptable behaviour. Violations of this nature will be reported to appropriate authorities.
- I have read and understood the above Statement of Understanding.


Patient's signature

Shirley E. Gornham, RN, MN
Mental Health Services
2 Fd Amb Medical Clinic
Garrison Petawawa
Health Care Professional

27-08-2019
Date

T56588965
Service Number

27 Aug 19
Date

- Notes:
- This form will remain in effect for a period of six months after date of signature.
 - This form may be revoked by patient at any time by request to the signatory Health Care Professional verbally or in writing.

Privacy Act Statement: The information you provide on this form is collected under the authority of the *Privacy Act* and will be used as our good and sufficient authority to disclose information for the purposes specified. This form will be maintained as part of your medical CF 2034 and will be retained in the Personal Information Bank DND PPE 810 - Medical Records and DND PPE 812 - Psychosocial Files identified in Infosource. For more information, visit infosource.gc.ca. Infosource is also available in Canadian Public Libraries.

PROTECTED B (When completed)

Canada

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Scan-Consent Document Date: 2019-08-27

Signed by : Ms Wanda C Ziebarth, CLERK, PT-MH, 2019-08-27 14:32:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Admin Note Document Date: 2019-08-20

GENERAL INFORMATION

As per new protocol, this mbr's post deployment booklet was brought to writer's attention due to affirmative answer in the suicidality question. Writer spoke to Mbr's psychiatrist who saw the mbr 1 hour ago and assured writer that this mbr was not currently at risk and there were no safety concerns.

No further action required by writer--mbr to attend regularly scheduled post deployment screening interview as scheduled on 18 sept.

Signed by : Ms Lisa Hallett, SOCW, PT-MH, 2019-08-20 14:40:00

END OF DOCUMENT



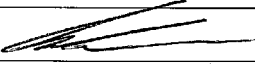
Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Scan-Reintegration Screening Document Date: 2017-11-09

PROTECTED B

CANSOFCOM REINTEGRATION SCREENING

Service Number :	Last Name :	First Name :
T56588965	Bardyla	Ryan
Unit : CSSR	Interviewer :	Date :
Rank : MCpl	Lt. CW Reid TFFA	09-11-17

Questions/Areas	Rate	Comments
Deployment experience	☒ G / ☐ B	Overall good deployment
Change in physical health	☐ Y / ☒ N	Wt. loss. able to exercise more
Energy	H ☒ M / ☐ L	Highs and lows
Concentration	H ☒ M / ☐ L	& changes from the usual
Mood	H ☒ M / ☐ L	mostly good
Exposure to traumatic events	☐ Y / ☒ N	UAV feed of dead IDPs, and engagements No concerns.
Sleep	☒ G / ☐ Ok / ☐ P	& issues falling or staying asleep
Nightmares	☐ Y / ☒ N	None
Stress symptoms	☒ H / ☐ M / ☐ L	Overall pt was able to manage. 2+ episodes of extreme mood swings.
Intrusive thoughts	☐ Y / ☒ N	
Changes in self	☐ Y / ☒ N	None
Home/Family	☒ G / ☐ Ok / ☐ B	Wife & 2+ kids Everything is good at home. & concerns
Significant challenges	☐ Y / ☒ N	
Previous reintegration	G / ☐ Ok / ☐ P	& prior re deployments.
Concerns about returning to work	H ☒ M / ☐ L	Uncsure of plan in 2018 year.
Any other concerns	☐ Y / ☒ N	
Follow-up required prior to EPDS	☐ Y / ☒ N	

Signature: 
C. Reid Lt. TFFA

PROTECTED B

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Scan-Reintegration Screening Document Date: 2017-11-09

Signed by : Ms Krista Lawson, CLERK, CSC-01 CDU1, 2018-02-06 08:20:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09

Scan-DND2957 Pre Deployment Screening

Document Date: 2017-01-12



PROTECTED B (When completed)
PROTÉGÉ B (Une fois rempli)

Pre-Deployment Screening
Sélection préliminaire avant déploiement

SN - NM T56588965	Rank - Grade MCpl	Last name - Nom de famille Bardyla	First name - Prénom Ryan		
MOS ID - ID SGPM 000362-3	DOB - DDN (yyaa-mm-dd) 1981-10-09	Gender - Sexe ☒ M ☐ F	Marital status - Statut matrimonial married		
Unit - Unité CSOR	Previous unit - Unité précédente 2 CMBG HQ and Sigs		COS date - Date du CE 14 July 2016		
Phone no. (office-bureau) 613-687-5514x6070	(home-maison) 613-685-0807	Partner's phone no. (office-bureau) 613-685-0807	(home-maison)		
Deploying to - Déploiement à		Deployment dates - Dates de déploiement			
Partner's name - Nom du/de la partenaire Jan Bardyla		Partner's employment - Emploi du/de la partenaire House Keeper			
Child(ren)'s name(s) - Nom du/des enfant(s) Liam Bardyla	Age - Âge 12	Gender - Sexe ☒ M ☐ F	Child(ren)'s name(s) - Nom du/des enfant(s) Logan Bardyla	Age - Âge 10	Gender - Sexe ☒ M ☐ F
Do these children normally reside with you? Ces enfants habitent-ils généralement avec vous?		Are you a single parent? Êtes-vous un parent célibataire?			
☒ Yes - Oui ☐ No - Non		☐ Yes - Oui ☒ No - Non			
To be completed by the social worker - À être complété par le travailleur social					
Have you had contact with a social worker, chaplain, medical officer, or staff of other counselling services within the last 5 years concerning a personal problem or situation? A cours des 5 dernières années, avez-vous pris contact avec un travailleur social, un aumônier, un médecin ou tout autre organisme social pour un problème ou une situation de nature personnelle?		(if yes, state who/when) (si oui, veuillez spécifier qui/quand)			
No - No Issue		☒ Yes - Oui ☐ No - Non			
Are you involved in any legal matter (being sued, awaiting court proceedings, obtaining a legal separation/divorce, involved in a child adoption or custody proceeding, buying or selling property, etc.)? Êtes-vous impliqué dans une affaire juridique (poursuite, recours en justice, obtention d'un divorce ou d'une séparation de corps, adoption, procédure de garde, achat ou vente d'une propriété, etc.)?		(if yes, state who/when) (si oui, veuillez spécifier qui/quand)			
		☐ Yes - Oui ☒ No - Non			
Have you completed a normal tour of duty outside Canada or a posting at an isolated location? Avez-vous effectué une période de service normale à l'extérieur du Canada ou une mutation dans un lieu isolé?		(if yes, state who/when) (si oui, veuillez spécifier qui/quand)			
No - None		☐ Yes - Oui ☒ No - Non			

DND 2957 (02-2014)

Design: Forms Management 613-995-9844
Conception: Gestion des formulaires 613-947-8944

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PROTECTED B (When completed)
PROTÉGÉ B (Une fois rempli)

Canada



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09

Scan-DND2957 Pre Deployment Screening

Document Date: 2017-01-12

PROTECTED B (When completed)
PROTÉGÉ B (Une fois rempli)

Do you have any immediate family located in public institutions (extended care hospitals, homes for the elderly or mentally challenged, etc.)? Avez-vous des personnes à charge vivant dans des institutions publiques (établissement de soins prolongés, foyers pour personnes âgées, centre pour personnes ayant une déficience mentale, etc.)?	(if yes, state who/when) (si oui, veuillez spécifier qui/quand)	☐ Yes - Oui ☒ No - Non
PARENTS - CHATMAN, CN SPOUSES - PARENT - SON - LIVING		
Is any member of your immediate or extended family suffering from a serious illness and/or is so sick that you feel you cannot be away from home at this time? Est-ce qu'un membre de votre famille immédiate ou étendue souffre d'une maladie grave ou est tellement malade que, selon vous, il vous serait impossible de vous absenter de votre foyer en ce moment?	(if yes, state who/when) (si oui, veuillez spécifier qui/quand)	☐ Yes - Oui ☒ No - Non
Are you legally, financially or morally responsible for anyone other than someone in your immediate family? Êtes-vous responsable, juridiquement, financièrement ou moralement, de quelqu'un en dehors de votre famille immédiate?	(if yes, state who/when) (si oui, veuillez spécifier qui/quand)	☐ Yes - Oui ☒ No - Non
Are you or is any member of your family experiencing any difficulty with substance abuse, other addictions or gambling? Est-ce que vous ou un membre de votre famille éprouvez des difficultés liées à l'abus d'alcool, de drogues, de médicaments, autres formes de dépendances, ou de jeux de hasard?	(if yes, state who/when) (si oui, veuillez spécifier qui/quand)	☐ Yes - Oui ☒ No - Non
Are you or any member of your family experiencing any psychological, emotional or social difficulties (child's school/ learning/behavioural problems, parenting issues, financial distress, etc.), which might require you to be returned home early from this deployment? Est-ce que vous ou un membre de votre famille éprouvez des difficultés d'ordre psychologique, émotionnel ou social (école/problèmes d'apprentissage ou de comportement, problèmes parentaux, difficultés financières, etc.) qui feraient en sorte de précipiter votre retour d'un déploiement?	(if yes, state who/when) (si oui, veuillez spécifier qui/quand)	☐ Yes - Oui ☒ No - Non
CHILDREN AT PINGVING, CN		

DND 2957 (02-2014)

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PROTECTED B (When completed)
PROTÉGÉ B (Une fois rempli)



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Scan-DND2957 Pre Deployment Screening Document Date: 2017-01-12

PROTECTED B (When completed)
PROTÉGÉ B (Une fois rempli)

Are there personal or family circumstances for which you should not deploy at this time? Vous trouvez-vous dans une situation personnelle ou familiale qui vous laisse penser que vous ne devriez pas être déployé en ce moment?	(if yes, state who/when) (si oui, veuillez spécifier qui/quand)	☐ Yes - Oui ☒ No - Non
The CF member and their partner are equally entitled to discuss any personal or family issue with a social worker. Do you wish to arrange an appointment? Le militaire et son/sa partenaire sont tous les deux autorisés à discuter d'un problème personnel ou familial avec un travailleur social. Désirez-vous prendre un rendez-vous?	(if yes, state who/when) (si oui, veuillez spécifier qui/quand)	☐ Yes - Oui ☒ No - Non

Privacy Act Statement / Énoncé concernant la Loi sur la protection des renseignements personnels

Personal Information collected on this form is used to determine or verify the suitability of an individual to serve in the Canadian Forces (CF). It is collected under the authority of the *National Defence Act* and the *Queen's Regulations and Orders* for the CF and is protected by the provisions of the *Privacy Act*. The completion of this form is mandatory. A refusal to provide information will lead to a review of whether the person is eligible to hold a position or is fit to perform duties as a serving member of the CF. The personal information collected is described in the DND Personal Information Bank PPE 812 Psychosocial Services. Instructions for obtaining this information are outlined in the government publication entitled Info Source.

Les renseignements personnels recueillis dans ce formulaire sont utilisés pour déterminer ou vérifier l'aptitude d'un individu à servir dans les Forces canadiennes (FC). Ils sont recueillis en vertu de la *Loi sur la défense nationale* et les *Ordonnances et Règlements royaux* applicables au FC et protégés par les dispositions de la *Loi sur la protection des renseignements personnels*. Il est obligatoire de remplir ce formulaire. Le refus de fournir des informations conduira à un examen à savoir si la personne est admissible à un poste ou apte à accomplir les tâches en tant que militaire actif des FC. Les renseignements personnels recueillis sont expliqués dans les fichiers de renseignements personnels du MDN, PPE 812, Services psychosociaux. Les instructions pour obtenir cette information sont décrites dans la publication gouvernementale intitulée Info Source.

Declaration - Déclaration

I declare that the responses given above pertaining to my being screened for deployment are true and accurate, to the best of my knowledge.
Je déclare que les réponses données ci-dessus concernant ma sélection préliminaire pour déploiement sont exactes et véridiques, au meilleur de mes connaissances.

Member's signature - Signature du membre	Date (yyaa-mm-dd)
Assessment - Évaluation FCP INVOLED NEIGHBOURS AND MILITARY. NO PREVIOUS TOURS MARRIED x 2 CHILDREN AT PREVIEW NOTE MARR SELF-REPORT AS GIVEN ON THE CONTINUUM ASSESSOR IS GOING TO DEPLOY	
Deployment status - Statut du déploiement ☒ G / V ☐ Y / J ☐ R	
Social worker (print) - Travailleur social (imprimé) A. Varga Maj	Signature [Signature] Date (yyaa-mm-dd) 2017-1-12

DND 2957 (02-2014)

CSOR Chap
813-687-5511 ext 4494

PROTECTED B (When completed)
PROTÉGÉ B (Une fois rempli)

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END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Scan-DND2957 Pre Deployment Screening Document Date: 2017-01-12

Signed by : Mr Robert JG Lacelle, CLERK, PT-PCS, 2019-08-09 13:53:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Scan-DND2949 Discharge Summary Document Date: 2015-06-09

PROTECTED B (when completed) / PROTÉGÉ B (une fois rempli)
MENTAL HEALTH - EXTERNAL PROVIDER DISCHARGE SUMMARY /
SANTÉ MENTALE - SOMMAIRE LORS DE LA CESSATION DES SERVICES DU FOURNISSEUR EXTERNE
GARRISON PETAWAWA/ GARNISON PETAWAWA

- ☐ For Sign Off by: _____
☐ For Review by: _____
☐ Scan to File only

SN / NS T56 588 965	Last name / Nom Bardyla	First name / Prénom Ryan	Rank / Rang Cpl
DOB / DDN 09-Oct-81	Telephone / Téléphone	Work / Travail: Home / Maison: 613- 685-0807	Unit / Unité 2HQ&Sigs
Discharge diagnosis (if applicable) / Diagnostic lors de la cessation des traitements (le cas échéant):		Therapy start date / Date du début du traitement Nov 12,2014:	
		Therapy end date / Date de cessation du traitement June 09,2015:	
		Name of GDMO (physician) / Nom du MMG (médecin):	

Reason for initial referral / Raison pour la demande initiale:

Cpl Bardyla was referred for chronic low mood, quick to anger which had affected relationship and peers and chronic worrying- request for anger management and diagnostic assessment of mental health and referred to author on Oct 20,2015.

Summary of treatment / Résumé du traitement:

Cpl Ryan Bardyla demonstrated mood issues of depression and anger, was very upset with how conducted himself to his 2 children and his wife who attended and came to a third meeting. She was aware his moods often needed delicate handling but was committed to the marriage as saw him begin to be more levelled out and happier- had forgiven an indiscretion on his part with 3rd party while tasked to Kingston last summer.
Despite initial resistance, did follow up with MO Moore for Zoloft rather than Welbutrin previously had tried.- also engaged in sobriety which he found helped.
He had increased self esteem when was named signaller of the year and focus in insight counselling re job and relationship continued. He was increased on his Zoloft to 100 mg by MO Moore from his report at meeting Apr 7,2015 after incident where reactive at work-helped a lot. Also his financial burden was assisted by wife Jan getting 2 part time jobs.
By last contact on June 9,2015 he was happier at work, had been promoted to MCpl on April 1st, was accepting his partner better and appreciating his overall situation in a more positive way. Jan had obtained better part time job at Miramichi, had less financial stress, were able to afford to follow up on recommendations for dates, felt happier and stronger commitment- felt the conversations had helped indirectly without him realising it- had a total of 9 meetings..

Reason for discharge / Raison de la cessation des services:

- | | |
|--|--|
| ☐ Treatment completed / Traitement complété | ☐ Deployed, tasked/ Déployé , service temporaire |
| ☐ Client withdrew from treatment / Le client s'est
désisté | ☐ Posted / Mutation |
| ☐ Released / Libération | ☐ Sessions missed _____/
Scéances manquées _____ |
| ☐ Assessment only / Évaluation seulement | ☐ Other / Autre: _____ |

PROTECTED B (when completed) / PROTÉGÉ B (une fois rempli)

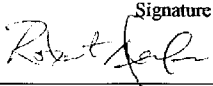


Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Scan-DND2949 Discharge Summary Document Date: 2015-06-09

PROTECTED B (when completed) / PROTÉGÉ B (une fois rempli)
MENTAL HEALTH – EXTERNAL PROVIDER DISCHARGE SUMMARY /
SANTÉ MENTALE – SOMMAIRE LORS DE LA CESSATION DES SERVICES DU FOURNISSEUR EXTERNE
GARRISON PETAWAWA/ GARNISON PETAWAWA

- ☐ For Sign Off by: _____
☐ For Review by: _____
☐ Scan to File only

Discharge plan (including needs left to be met and plan for client to meet these needs) / Plan lors de la cessation des services (y compris les besoins non-satisfaits et le plan pour y répondre):

Name of clinician (print) / Nom du clinicien (lettre moulées)	Occupation / Profession	Signature	Date
Robert Neilson	Social worker		2015-06-09

PROTECTED B (when completed) / PROTÉGÉ B (une fois rempli)

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09
Scan-DND2949 Discharge Summary Document Date: 2015-06-09

Signed by : Jaime J Macleod, CLERK, PT-MH, 2015-07-07 08:32:00

Signed off by : Ms Shirley E Gorham, RN, PT-MH, 2015-07-07 16:22:00



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09

Scan-DND2411 Assessment of Psychosocial Functioning

Document Date: 2014-11-12

INITIAL ASSESSMENT

T56 588 965

NAME: Ryan Bardyla

DATE: Nov 12, 2014

PRESENTING PROBLEM AND HISTORY:

Cpl Bardyla was assessed by MO Moore on Sept 29, 2014 for chronic low mood, quick to anger which had affected relationship with wife and peers, and chronic worrying with referral for psychosocial treatment of anger management and diagnostic assessment at mental health. Warrior Support Services referred to author on Oct 20, 2014.

RELEVANT SOCIAL HISTORY:

Ryan is age 33 years, a Cpl, an information systems specialist in 5 years in February 2015, has signed up for another contract. He was a licensed electrician before joining up, thought he would be doing more industrial electrician work, finds job boring; mostly fixes radios and all other communication equipment. He convinced his wife to let join up as he wanted to do "cool" things, if not like would get out after 4 years, has not done so.

Ryan has been with Jan, age 35 years for 10 years, married 7 years, have 2 children ages 8 and 10 years. She recently has started 2 part time jobs. His family of origin is in Chatham, had a cut off with mother until recently; also has brother and sister in Chatham. His marriage is going all right now, was incident; sorted out without professional help, trust is back.

Ryan reported was seen by Dr Roy who did an unidentified questionnaire with him, indicated was above threshold for depression, too critical of self, quick to anger, projects on to others, agitated all the time, and worries. He reported that Dr Roy was recommending medication to MO Moore, needs to set up appointment there.

Ryan reported that he was on medication from ages 10-15 years, was picked on and bullied whole life, not have good personal image, indicated does have personal confidence but a negative attitude.

CLINICAL IMPRESSION:

Ryan focused meeting on relationship with his wife Jan, described her as passive person very patient with him, she does everything, not ask for help, "not want to poke the bear". He spoke to fact that he does not treat her with respect; said he cannot control his mouth. He feels the children are well treated but he isolates self downstairs, stems from wanting to be better for them.

Ryan said he is willing to help with family responsibilities more, help kids with homework, do more outside; have more quality time with them. He also knows he talks to Jan in disrespectful way he models, does not want children to emulate, wants them to have functional relationship with mother and others. He has a concern re how he tones down, has tried but wants to be more approachable.

Since he focused at length on family matters, it was suggested he bring his partner to meeting to provide therapist more context for his own observations, may involve some couples counselling also. He also will be probably using medication to assist with mood issues of depression and anger. Strategies non medical to deal with them can be also developed.

TREATMENT PLAN:

Page 1 of 2



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09

Scan-DND2411 Assessment of Psychosocial Functioning

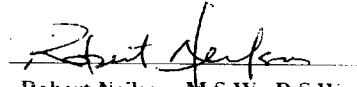
Document Date: 2014-11-12

INITIAL ASSESSMENT

NAME: Ryan Bardyla

DATE: Nov 12, 2014

Individual treatment for the aforementioned and possible limited couples counselling is recommended.


Robert Neilson, M.S.W., R.S.W.

Page 2 of 2

END OF DOCUMENT



Patient: BARDYLA, RYAN, MCpl, T56588965 DOB: 1981-10-09

Scan-DND2411 Assessment of Psychosocial Functioning

Document Date: 2014-11-12

Signed by : Mr Howard G Palmer, CLERK, PT-MH, 2014-11-25 13:14:00